From Model to Practice:
The Making of American Hospice

A thesis presented

by

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to

the Department of the History of Science
in partial fulfillment of the requirement for an honors degree in
History and Science

Harvard University
Cambridge, Massachusetts
March 2026

ABSTRACT

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Abstract: This thesis examines the development of the modern hospice movement through a case study of Connecticut Hospice in New Haven, Connecticut. This hospice, the first in the United States, emerged in the context of the growing discourse surrounding dignity in death and care for the dying in the 1950s and 1960s, and the creation of modern hospice in London under the direction of Cicely Saunders in 1969. This analysis shows the increasing establishment and authority of this American hospice model of care, and the subsequent integration into the American medical system. Tracing this trajectory, this thesis explores the tension between institutional bureaucracy and the humanistic reform impulses that gave rise to the American hospice movement.

Keywords: hospice; modern hospice movement; Connecticut Hospice; terminal care; end-of-life care; healthcare reform; medicalization of death

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Acknowledgements

Thank you to the History of Science department and the brilliant faculty that have made my experience in this department so formative: Joelle Abi-Rached, Naomi Oreskes, Simon Torracinta, Allie Belser, Erik Baker, Alex Csiszar, Peter Gallison, Anne Harrington, and so many more. I am so glad to have found this expansive and wonderful department so full of curiosity and care.

To my thesis advisor, Maud Jansen, thank you for your guidance through this exhausting process. Thank you for your honesty about every incoherent thought, your advice through every moment of procrastination, and your presence through every early morning dining hall meeting. I will forever be in awe of your seemingly endless knowledge of all things aging, hospice, and palliative care, and I feel lucky to have gotten to pick your brain. Thank you.

Thank you to my family for your endless encouragement and love. To my mom, thank you for being my cheerleader and informal second advisor. It is only because of you that I can string a sentence together, let alone this many in a row. Dad, thank you for keeping my spirits up with pictures of Tucker and the girls, and the Codman trips. Thank you both, from the bottom of my heart.

Kwee, thank you for your contagious excitement. You consistently reminded me of my passion for this project and bolstered the flames when they got low.

Thank you to Rachel for commiserating, celebrating, and rotting with me. Rachel, my thesis companion, what would I have done without you?

Thank you to Em and Lorelei, for being my support from afar. Em, your advice and tales of survival kept me going through many late nights.

And thank you to Barbara Pearce for welcoming me to Connecticut Hospice and introducing me to the GUIDE and Palliative care team. Amy, Fiona, Jen, Mark, Angel, and Paula, thank you for being my introduction to everything hospice care can be.

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Introduction

Death is the one certainty in life, and the humanity's history of reckoning is storied and long.[1] However, care for the dying was not an established medical specialty in the United States until well into the twentieth century.[2] Increasing medicalization and expanding technological developments across all American health care fields in the twentieth century created a focus on curing disease, not on the holistic patient.[3] Consequently, patients with "incurable" or terminal illness were essentially abandoned, left to cope with the physical and emotional aspects of terminal illness alone.[4] The modern hospice was the response to this neglect and inadequacy of care.

This thesis focuses on a group of people in Connecticut who started the first hospice in the United States, later known as Connecticut Hospice but founded under the name Hospice, Inc. Hospice is a primary approach to end-of-life care that is now fully institutionalized and integrated in the national healthcare and insurance system in the United States. However, the establishment of hospice was a long process that occurred rather late in the context of American medical systems. Florence Wald and her colleagues headed this effort by creating a new model of care consisting of new approaches to pain, an interdisciplinary care team, and an emphasis on interpersonal relationships. American hospice became established and integrated into the larger healthcare system after significant work spreading the word and proving the necessity of the emphasis on caring, not healing.

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[1] Allan Kellehear, *A Social History of Dying* (Cambridge University Press, 2009).
[2] David Clark, *Cicely Saunders: A Life and Legacy* (Oxford University Press, 2018), 311.; Eric J. Cassell, "Dying in a Technological Society," *The Hastings Center Studies* 2, no. 2 (1974): 31, https://doi.org/10.2307/3527480.
[3] Emily Abel, *The Inevitable Hour* (Johns Hopkins University Press, 2017), 105, https://doi.org/10.1353/book.22169.
[4] Abel, *The Inevitable Hour*, 5.

I center the formation of Connecticut Hospice with three key – and overlapping – periods of development in my three chapters. By tracing the development of hospice in New Haven and the evolution of the hospice model of care, I gain insight into the integration of the movement into the existing American healthcare system. Chapter One explores the creation of the original hospice model of care, inspired by the emergence of hospice in the United Kingdom, headed by physician Cicely Saunders, and a research study of the needs of the dying conducted by founding members of Hospice Inc. After establishing the Home Care program and beginning to provide care to local patients, Chapter Two details how this new organization began refining their institutional reputation and garnering support for the expansion into a physical facility. Chapter Three traces how Hospice, Inc., worked to publicly justify the need for their model of care, to spread their model of care, and to instruct others through education initiatives on how to provide their model of hospice care.

Chapter One opens with the cultural dissatisfaction surrounding the medical management of death in the 1960s and 1970s: what contributed to the impression that a special kind of care was needed for the dying and terminally ill? I argue that the American model of hospice care was heavily influenced by the British model of hospice, but departed in key ways to account for differences in the healthcare system and population. I begin my analysis with a brief discussion of the emergence of modern hospice in a movement headed by Saunders in the United Kingdom in 1969, her approaches to pain management, and her view of death as a psychosocial phenomenon. I discuss how Saunders and her ideas traveled to the United States, and how Florence Wald and her colleagues in New Haven came together around the idea of hospice in America. Through analysis of internal documents from the Hospice Planning Group and their foundational study of dying patients, I draw conclusions

about the influence of Saunders' writings and experience on the original American hospice philosophy that would support the foundation of the Home Care Program and the future in-patient facility at Hospice, Inc. I argue that, while Wald and her colleagues were very inspired by Saunders' work, they were careful to adapt her philosophy of care to their community and the needs of the local patients, creating a uniquely American model of hospice care.

My second set of historical questions revolves around community response and the creation of a physical place in which to render hospice care: why did Hospice, Inc., need a facility distinct from the hospital? How did public response and funding impact this process of development? I argue that Hospice, Inc., focused especially on its reputation and public image due to the significant financial and political support needed to open the in-patient facility and truly establish itself in the healthcare ecosystem as a credible and respected institution. I analyze the Hospice, Inc., newsletter, and the significant news coverage that the concept and the new Home Care model received during this time. Using the perspectives of the local Connecticut community and the surrounding American context, I argue that the Hospice, Inc., founders were leveraging their philosophy to fit the unfulfilled needs of the dying and their families, as well as of the physicians and hospitals, to gain the support they required.

Finally, I explore the process of popularization by which Hospice, Inc., made arguments for the necessity of the hospice model of care, spread their model of care, and educated providers on how to practice this model of care. I argue that the progression of increasingly involved publicity efforts reflects the increase in authority that Connecticut Hospice was able to gain on the issue of care for the dying and terminally ill. Through

analysis of speaking engagements, workshops, and curriculum that the Hospice, Inc., founders created, the evolution of the hospice theory from its beginnings becomes very clear in its simplification. The original hospice philosophy was idealistic, but became increasingly simplified and, particularly in their educational initiatives, significantly more grounded in the realities of the healthcare system, confirming their widespread integration and establishment.

Positioning my own research within existing historiography, I found that most of the literature on the hospice movement covers the founders as revolutionaries and pioneers, focusing on their individual motivations and personal experiences. I contribute to the field of the history of medicine by foregrounding the model of care that the first American hospice instantiated, examining the institutional design of hospice, and contextualizing how this model of care responded to the existing landscape of healthcare. To do so, I draw from archival materials of the early development of Connecticut Hospice.

Emily Abel wrote two books on death and hospice in America: *Prelude to Hospice* and *The Inevitable Hour*. The most prominent book on Connecticut Hospice*, Prelude to Hospice,* is a historical analysis of the study that Florence Wald and her colleagues conducted on the needs of dying patients and their families.[5] The opening of Hospice, Inc., is only discussed in the book's conclusion, as the title suggests.[6] Abel uses the Florence and Henry Wald Archive housed at Yale University to provide insight into Wald as a character and practitioner of hospice. Abel argues that, amidst Wald's good intentions and the difficulty/chaos of terminal care, Wald created a vision of hospice that was idealistic and unsustainable. I use the same archive as my primary source base, but, in contrast, I use it to

[5] Emily K. Abel, *Prelude to Hospice: Florence Wald, Dying People and Their Families*, Critical Issues in Health and Medicine (Rutgers University Press, 2018).
[6] Abel, *Prelude to Hospice*.

gain insight into the creation of the hospice model of care and subsequent influence of Hospice, Inc., as a source of hospice knowledge, rather than to understand or analyze Wald and her interpersonal motivations and conflicts. Abel's book does, nonetheless, inform my analysis of the context of Connecticut Hospice's origins and provide a guide for highlighting important figures and aspects of their personal lives. Abel's other book, *The Inevitable Hour*, covers the history of end-of-life care in America from the nineteenth century through 1965. Abel's book provided much baseline understanding, from which I built my arguments for this thesis. Building on the framework of Abel's research, I strive to expand this body of work by extending the timeline of analysis and focusing on the developing institution.

David Clark – another central hospice historian and the leading scholar of the founder of English hospice, Cicely Saunders – brings an interdisciplinary sociological perspective to the historiography of hospice. His scholarship focuses on the evolution of early ideas of hospice from the nineteenth century through to the modern hospice movement starting in the late 1960s, and it highlights cultural, medical, and ethical shifts in hospice and its overlap with palliative care.[7] Adding to Clark's broader discussion of the international development of hospice care, I contribute a closer focus to one foundational institution in the United States.

Hospice continues to gain importance, as the life expectancy of the American population rises, and as medical technology continues to develop and flourish. Given the ever-increasing variety of treatment options, the question of when to stop treatment will

[7] David Clark, *To Comfort Always: A History of Palliative Care* (Oxford University Press, 2016); David Clark, *A Bit of Heaven for the Few? : An Oral History of the Modern Hospice Movement in the United Kingdom* (Mortal Press, 2003); David Clark, "`Total Pain', Disciplinary Power and the Body in the Work of Cicely Saunders, 1958–1967," *Social Science & Medicine* 49, no. 6 (1999): 727–36, https://doi.org/10.1016/S0277-9536(99)00098-2.

become a more common and more difficult debate. Hospice is the current system through which we manage this decision and the care that follows. Understanding the evolution of this model of care, the original intentions, and the execution of those ideals, is imperative for contemporary analysis and growth. Furthermore, hospice is a prime example of modern medical reform. As our healthcare system continues to grow and improve, understanding past processes of evolution and improvement will support our continued growth and reform. This thesis aims to contribute to this contemporary discussion and deepen understanding of the beginning of hospice in the United States.

Chapter One

Transplanted or Translated: The European Roots of American Hospice

Hospice was not an American idea. It originated as we now know it in England in the mid 1900s as a passion project by nurse and almoner later turned doctor, Cicely Saunders. Her creation of St. Christopher's Hospice was an influential development for the coming international adoption of modern hospice. Florence Wald was one of the major figures who brought this concept of new and improved hospice care from Britain to the United States. However, given the differences in healthcare system structure, lack of national healthcare coverage, and cultural differences, hospice in the US would not look the same as it did in England. While Saunders found her calling through reforming pain management techniques for the terminally ill and the dying, the American hospice movement—building on Saunders' foundation—focused more intensely on community relations and the challenge of integrating hospice into the existing American medical system.

This chapter aims to investigate how the English concept of hospice was brought to American soil and the following creation of the three main tenets of the American hospice model of care: the psychosocial components of death and pain, the importance of open communication with patient and family, and adaptation to circumstance. I will extend Emily Abel's analysis in *Prelude to Hospice* by foregrounding the creation of the American hospice model of care. It is not a moral critique of Wald, nor an idealized approach to what hospice should have been or could be[8], it is an examination of hospice in its origins. Extensive use is

[8] Ara Francis, "Death Positivity, White Respectability: A Critical History of Cicely Saunders and the Hospice Model of Care," *Critical Sociology*, June 26, 2025, 08969205251350396, https://doi.org/10.1177/08969205251350396.

made of the unpublished working papers, planning documents, newspaper clippings, and correspondence of Florence Wald held at this archive. Published material, as well as historiography on the modern hospice movement and the involved figures, is also included.

European Roots: Cicely Saunders and St. Christopher's Hospice

Before the modern movement, hospice existed in Europe within catholic organizations run primarily by nuns to care for the poor.[9] These hospices began emerging in the latter half of the nineteenth century following outbreaks of tuberculosis (often called consumption during this time) and cholera. The poor were left with two options to cope with these largely fatal diseases: die in their crowded tenement (slums) housing or try to get care in an infirmary or almshouse.[10] Neither option was attractive, as dying at home meant little to no medical care and put families at great risk, and infirmaries were dirty, overcrowded facilities often attached to workhouses that were short-staffed and largely unable to provide any meaningful treatment to their patients beyond a roof over their head.[11] Recognizing the need for a better source of care, sisterhoods of nuns began forming facilities to provide better circumstances for individuals who were beyond treatment and simply needed a place to die with dignity. One such sisterhood significant to Cicely Saunders' journey with caring for the dying, was the Sisters of Charity who, at the request of a Cardinal and a Father, created St. Joseph's Hospice in 1905 in East London.[12] While the nuns were gathering the resources to secure a physical location, they did home visits to provide emotional and limited medical

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[9] Abel, *The Inevitable Hour*, 30.
[10] Charles E. Rosenberg, *The Care of Strangers* (Johns Hopkins University Press, 1995), 4, https://doi.org/10.56021/9780801850820.
[11] Rosenberg, *The Care of Strangers*, 29.
[12] Clark, "`Total Pain', Disciplinary Power and the Body in the Work of Cicely Saunders, 1958–1967," 729."Our History - The History of the Hospice," St. Joseph's Hospice, https://www.stjh.org.uk/about-us/our-history/.

care to the dying, creating echoes to the home care programs that would later develop in modern hospice. Within two years of opening, this facility was prompted to expand their bed capacity due to high demand.[13] Several other similar facilities popped up in Britain, Scotland, and Ireland during this time, all started by nuns. These facilities specialized in basic patient care, spiritual comfort, and dignity in death, and while these facilities were the first specialized care for the dying, they did not mark the beginning of the *modern* hospice movement, though they lay the critical foundation. This modernity came with Cicely Saunders' facility, St. Christopher's founded in 1967, and the solidification of the hospice philosophy of integrating pain management, psychological, social, and spiritual care.

Cicely Saunders, an innovator who has received much attention for her creation of St. Christopher's Hospice (the first *modern* hospice) and the concept of "total pain," is largely credited as the founder of the modern hospice movement.[14] Saunders' professional career as a nurse, a social worker, and then a physician, gave her unique depth of insight into many of the specific issues facing the dying. A prolific writer, she published many proposals, articles, and books over the years, outlining her experiences in caring for those with chronic and terminal illness and her unique techniques for pain management, using morphine and other opiates rarely used in the care of the dying.[15] Her recognition of inadequacies and gaps in care for the dying began early in her time working in hospice care. After receiving her

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[13] Clark, "`Total Pain', Disciplinary Power and the Body in the Work of Cicely Saunders, 1958–1967," 729.
[14] Clark, "`Total Pain', Disciplinary Power and the Body in the Work of Cicely Saunders, 1958–1967," 730.; Clark, *Cicely Saunders: A Life and Legacy*, 311.
[15] Cicely Saunders, *Hospice and Palliative Care: An Interdisciplinary Approach* (Hodder Arnold, 1990).; Cicely Saunders, *Hospice and Palliative Care: An Interdisciplinary Approach* (Hodder Arnold, 1990); Cicely Saunders, *Living with Dying: A Guide to Palliative Care* (Oxford ; New York : Oxford University Press, 1995); Cicely Saunders, *Beyond All Pain: A Companion for the Suffering and Bereaved* (SPCK Publishing, 1983); Cicely Saunders, *The Management of Terminal Disease.* (Edward Arnold, 1978); Cicely M. Saunders, *Cicely Saunders : Selected Writings 1958-2004*, with Internet Archive (Oxford ; New York : Oxford University Press, 2006), http://archive.org/details/cicelysaundersse0000saun; Cicely Saunders, *Dying of Cancer*, 1958.

doctors qualification, Saunders began her work as a physician at St. Joseph's hospice, where she would perform her studies of the dying, experiment with pain management techniques, and reflect on the role of the provider in end-of-life care.[16] Her experiences at this institution were formative for the development of her idea of "total pain," and for her vision of what would become St. Christopher's hospice, as shown by her extensive writings on her time spent at that institution.[17] She recognized this concept she called 'total pain' as an all-encompassing wrongness and discomfort, one that was not being addressed by the care standards of that time, and leaving the dying in significant emotional and physical discomfort.[18] In one article, entitled "Dying of Cancer," she outlines the problems faced by dying patients in Britain, emphasizing a shortage of facilities and beds in, "hospital[s] or special Home[s] or Nursing Home[s] … accommodation of this kind is limited," [19] and a lack of freedom and time for practitioners to provide the level of attention required: "Panel Doctors are often not able to care as they would like for patients in continual pain and discomfort because of the demands on their time."[20] Following her experimentation, observation, and subsequent publications, her vision began culminating in the proposal for St. Christopher's, entitled "The Scheme," published in 1959. When St. Joseph's began focusing in on chronic illness (as opposed to terminal illness), Saunders decided that it was time to leave St. Joseph's and begin putting the discoveries she made there towards her creation in earnest, as shown in a letter to colleague and friend Reverend Whitney Hale of New Hampshire, US:

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[16] Clark, *Cicely Saunders: A Life and Legacy*, 106.
[17] David Clark, *Cicely Saunders – Founder of the Hospice Movement: Selected Letters 1959–1999* (Oxford University Press, 2005), 9, https://doi.org/10.1093/acprof:oso/9780198569695.001.0001.
[18] Clark, "`Total Pain', Disciplinary Power and the Body in the Work of Cicely Saunders, 1958–1967."
[19] Saunders, *Dying of Cancer*, 4.
[20] Saunders, *Dying of Cancer*, 4.

> Certainly the end of the time at St Joseph's was shown very clearly, not least in the fact that for the first time since I have been there, … I find that there is not one single patient about whom I feel the kind of responsibility that makes me feel most unhappy about going away for a holiday. They will manage all right without me.[21] [22]

The intense personal relationships Saunders had with patients that she alludes to here (ones that became of a romantic and partnership nature twice) shows the complete investment of herself, both professional and emotional, into the work. Once this emotional and responsible feeling faded for the patients at St. Joseph's, she knew her care was no longer needed. Whether or not it is a coincidence that Florence Wald also had similar deep emotional relationships with patients and their families, often going beyond the depth of typical provider-patient relationships, is impossible to know, though, this pattern of total involvement is not to be overlooked and is not surprising given the intensity of dedication to this field and of emotion at this stage of life.

In "The Scheme," Saunders lays out her plans to "found a Home where the nursing staff will not have acute emergencies to cope with, nor complicated treatments, but will spend their time making their patients comfortable." [23] In this way, St. Christopher's was designed to be significantly different from typical acute care settings. This facility was to

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[21] David Clark, "Realizing a Vision (1959–1967)," in *Cicely Saunders – Founder of the Hospice Movement: Selected Letters 1959–1999*, ed. David Clark (Oxford University Press, 2005), 89, https://doi.org/10.1093/acprof:oso/9780198569695.003.0002.

[22] In a letter from Saunders to Reverend Whitney Hale in September of 1965: "I have been there [St. Joseph's] seven years now and as they are having many more chronic sick patients and fewer dying and as I have to get some writing done as well as all the work of St Christopher's, Reverend Mother and I have decided that now is the time for us to part." Clark, "Realizing a Vision (1959–1967)," ed. Clark (Oxford University Press, 2005), 89.

[23] *Archives of Dame Cicely Saunders (1918-2005): Cataloguing the Papers of the Modern Hospice Pioneer: Image*, n.d., accessed January 5, 2026, https://cicelysaundersarchive.wordpress.com/wp-content/uploads/2016/06/1_2_30_003.jpg.

continue the religious tradition of care while also supporting up-to-date medical care and research, with emphasis on community, home, and clinic. Saunders spent significant time considering minute details, with emphasis on the importance of closely monitored pain control, light–filled rooms, ease of transporting patients from room to room, dayrooms with comfortable chairs and cozy fireplaces, and an overall home–like atmosphere. Saunders emphasized that St. Christopher's would be intended to comfort not only the patients but their visiting families as well.

Saunders' nursing school alma mater (St. Thomas's) provided her with a grant in 1962 to travel to the US to study their treatment of the dying.[24] Thus followed several tours of the US, wherein Saunders was able to connect with many parties interested in death and dying including: the American Cancer Society, the National Institutes of Health, Herman Fiefel (the father of thanatology, the study of death), and, Florence Wald (see interactive map in footnotes).[25] On one of these tours in 1963, Saunders spoke at Yale Medical School. She would later return for a longer series of lectures at the Yale Nursing School at the request of Florence Wald. Though Florence Wald missed Cicely Saunder's first lecture at Yale Medical School[26], they became avid pen pals and eager colleagues.

Dying in America Before Hospice

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[24] David Clark, "Realizing a Vision (1959–1967)," in *Cicely Saunders – Founder of the Hospice Movement: Selected Letters 1959–1999*, ed. David Clark (Oxford University Press, 2005), 52, https://doi.org/10.1093/acprof:oso/9780198569695.003.0002.

[25] Interactive map of Cicely Saunders' travels across the US here.; Chris Olver, "StoryMap: Cicely Saunders' USA Tour, 1963," *Archives of Dame Cicely Saunders (1918-2005): Cataloguing the Papers of the Modern Hospice Pioneer*, September 14, 2015, https://cicelysaundersarchive.wordpress.com/2015/09/14/storymap-cicely-saunders-usa-tour-1963/.

[26] Abel, *Prelude to Hospice*, 15.

An understanding of the American climate is needed in order to understand the reception of Saunders' talks. I begin by examining the professional medical context and some of the factors that led to the climate of end-of-life care at that time. I then identify how their recognized inadequacies, ideals, and dissatisfactions shaped the imagined American hospice facility.

In the context of increasing specialization of medical practice there was no death and dying specialty, leaving these patients lacking personalized care. In his book, *The Social Transformation of American Medicine*, Paul Starr outlines the shift from individualized, self-reliant Americans to dependency on professionals and increasing specialization that paralleled the rural-to-urban shift that the country saw during the first half of the twentieth century.[27] As urban networks grew, health systems established themselves, creating complex systems of hospitals, salary physicians, insurance companies, and eliminating independent contractors. By the mid-1920s, the medical system had established organizational structures and cemented professional dominance and autonomy.[28] Due to quickly developing medical technology, treatment options and curative measures became the priority for the medical community, leaving behind patients deemed "incurable."[29] By the 1960s, it became clear that a new specialty was needed: care for death and the dying. Death occupied a central place in academic circles, marked by breakthrough publications such as Elizabeth Kübler-Ross's *On Death and Dying* in 1969, introducing the five stages of grief. Florence Wald and her colleagues were tuned into this widespread cry for reform, for *something.* Wald and her colleagues shared the view that one of these gaps in American healthcare was the insufficient

[27] Paul Starr, *The Social Transformation of American Medicine* (Basic Books, 1982), 24.
[28] Starr, *The Social Transformation of American Medicine*, 27.
[29] Abel, *The Inevitable Hour*, 105.

attention to end-of-life care—particularly a lack of pain management and attention to quality of life for the dying.

Humans have dealt with death from the beginning, so why did the gap emerge in this period and why not earlier? Dying in the United States had become increasingly medicalized and traumatic in the twentieth century.[30] For most ill patients, there was an abundance of curative resources, including cancer hospitals, research institutions, and oncologists. However, once a patient was deemed terminal and incurable, there were few options for palliation and no standard of care for symptom management in their final days.[31] Acute care settings were inappropriate due to the aggressive, curative treatments and lack of procedure for patient autonomy over care decisions. Additionally, there was no protocol for administering medication to manage the painful process of dying.[32] Death, often described as "failure" [33] by medical professionals, and taught as such in medical schools around the country, was a taboo and feared stage of life, one that the medical industry was unable to handle. Formerly, in the absence of the curative treatments, death was not something to be avoided and was accepted as a part of life. With the advent of effective cures, and thus control, death took on a new level of taboo. A lack of care and attention quickly developed within this taboo. Saunders' developments in Britain had begun the work of filling this gap, and thus it was time to bring these British ideas to American soil.

The Hospice Research and Planning Group Assembles

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[30] Kellehear, *A Social History of Dying*, 151.
[31] Abel, *The Inevitable Hour*, 120.
[32] Abel, *The Inevitable Hour*, 26.
[33] Abel, *The Inevitable Hour*, 106.

Florence Wald, a well-respected nurse, the dean of the Yale Nursing School[34], professor of psychiatric nursing, and an important contributor to the evolution of nursing education[35], was introduced to the work of Cicely Saunders by her colleague Virginia Henderson, who had attended Saunders' lecture at the medical school.[36] Wald and Henderson shared an interest in death and dying, and though Henderson would not go on to be involved in American hospice, her role in introducing Wald and Saunders cannot go unappreciated. Wald's fascination with Saunders' work was so great that after inviting her back to the US for a talk and ongoing written correspondence, Wald took a sabbatical in 1967 to volunteer at St. Christopher's. Following her time in London, Wald was inspired to begin her own investigation into the dying and their needs.

Wald gathered colleagues to accomplish this study, including Morris Wessel, a well-loved pediatrician in the New Haven area interested in end-of-life care; Edward Dobihal, a chaplain at Yale-New Haven Hospital with an interest in bereavement; and Ira Goldenberg, a doctor and oncologist who later became the primary physician for Connecticut Hospice. Together, they conducted "An Interdisciplinary Study of the Care of Dying Patients and Their Families," starting in 1968.[37]

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[34] Her career as Dean of the Nursing School would last from 1959 until 1966.

[35] Wald played a major role in transitioning the Yale School of Nursing from giving baccalaureate degrees to giving Master of Science in Nursing degrees, shifting the focus of the school and furthering the development of nursing as a career. This shift occurred in 1956, and the new curriculum was adopted in 1959 (coinciding with Wald's start as dean).

[36] Abel, *Prelude to Hospice*, 15.

[37] MS 1659, box 7, folder 89, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

This research group went on to form the Hospice Planning Group in 1971. The diversity of perspective and professions included in the group was intentional and internally celebrated. As they outline in the minutes from their first meeting:

> The professional and scientific knowledge of nursing and medicine combined with this reverence for life and its spirit serves to help the staff understand the experiences of the patient and his family and to relieve their distress. The philosophy of the projected Hospice evolves from our personal experiences with life and death.[38]

The makeup of this original planning group reflects the large clergy presence found on the board of St. Christopher's Hospice. This new and improved Planning Group expanded their initial objectives to include hiring and training a core staff for their own hospice program, launching a coordinated home-care program, and preparing for a future inpatient unit. [39]

Together, this group of individuals joined together by their interest in death and dying and their proximity to New Haven, began conducting a study based on an unfinished project of Wald's called the "Nurses Study of the Dying Patient and Their Families." This new effort began in 1968 and was soon renamed, "The Interdisciplinary Study of the Care of Dying Patients and Their Families." The varied group of nurses and physicians entered existing institutions to observe and support dying patients, speaking with patients and providers about their care and qualms, learning advanced cancer symptom management, and connecting with patient families.[40] Patients were referred to the study through board-member connections to

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[38] Hospice Planning Group Meeting Notes 1971. MS 1659, box 7, folder 89, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.
[39] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972 - December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 1.
[40] Hospice Planning Group Meeting Notes 1971. MS 1659, box 7, folder 89, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

local oncologists, and were given "special" personal care as incentive.[41] After two years of close observation, the research group and subsequent planning group defined the problems facing the dying and their loved ones as a generalized inadequacy of care. They described the state of care for the dying as "solitary, mechanized, inhuman, and often gruesome," occurring in nursing homes that were "ill-equipped and understaffed at best," or intensive care units where endless procedures and curative measures were used to "maintain life at any cost."[42] Their findings, summarized in "The Development of a Core Staff for Patient Care," argue that "modern society has become aware that its ways of dealing with the last stages of life, dying and death are inappropriate—even medically."[43] They discovered that only institutions focused on aggressive treatment would accept dying patients; chronic disease hospitals rarely admitted cancer patients needing more than minimal supportive care; and physicians avoided home visits. Family members often lacked the training or resources needed to care for loved ones at home. Even professionals supposedly trained to work with the dying were trained primarily in curative rather than supportive care. These were all less than optimal conditions for palliation. An optimal care facility did not exist for these patients, and even in the inappropriate facilities, helpful supportive care was not provided or even possible. This led to the dying experiencing loneliness, a lack of ability to cope with their mortality and impending death, feelings of abandonment, and deep discomfort – both physical and emotional.[44]

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[41] Abel, *Prelude to Hospice*, 3.
[42] Manuscript, "The Case for Support of Hospice, Inc." MS 1659 box 7, folder 8, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 3.
[43] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 5.
[44] "The Case for Support of Hospice, Inc." MS 1659 box 7, folder 8, Florence and Henry Wald Papers.

In addition to the impact on the dying, the research group identified difficulties that loved ones faced as well. These individuals often didn't know how to comfort their dying loved one in their last moments nor confront the death of their loved one afterwards. In a 1976 letter to Wald, a widow wrote describing her husband's death in 1969, saying "no one would really talk with us about what was happening. The only advice we received was, 'take one day at a time.' When you have very few days and the quality of those days is not good, this seems to me to be very poor counsel."[45] She goes on to comment on Hospice, Inc., by saying: "bless you for what you are doing. I know it helps."[46] After collecting an understanding of the factors negatively impacting both the dying and their families, the research group began planning a facility that would address these issues and protect the "right to die with dignity and enjoy life until the end."[47]

Tenets of American Hospice Care

Following the release of the study conclusions, the Hospice Planning Group released the "Philosophy Statement" in April 1971. Developments quickly followed: Hospice, Inc., was incorporated that November of 1971; and by February 1972, the New Haven Foundation had funded the hospice planning group. Using the data gathered from their study and advice from Saunders, the Planning Group began creating a set of tenets defining their envisioned model of care.[48] Three primary ideals emerged: a central psychosocial understanding of

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[45] Emily Henderson to Florence Wald, June 22, 1976, Box 7 folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[46] Emily Henderson to Florence Wald, June 22, 1976, Box 7 folder 73, Florence and Henry Wald Papers.
[47] Manuscript, "The Case for Support of Hospice, Inc." 1659 box 3, folder 25, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[48] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.; Clark, "`Total Pain', Disciplinary Power and the Body in the Work of Cicely Saunders, 1958–1967," 731.

death, dying, and pain; the importance of open communication with both patient and family; and care adaptable to circumstance and environment. In addition, several other aspects featured prominently in their planning documents: the question of institutional religiosity, scope of care, and staff hierarchy. This model of care would be a flexible system focused on rounding out patients' lives, offering a less expensive alternative to inpatient hospital care, providing a home-based model when possible, and prioritizing relief of physical, emotional, spiritual, and social distress.[49] The decentralizing of care provision from established institutions and healthcare systems sets it apart from most other medical specialties at the time, including more exclusively physician-oriented specialties like geriatrics and gerontology.

The Planning Group's analysis of pain management in hospital was critical, and they rejected the common practice of separating emotions and illness. Embracing Saunders' concept of "total pain," their acknowledgement of the impact of "fear of pain" in overall patient comfort and wellbeing influenced their departure from typical pain relief techniques. Instead of increasing pain medication gradually, they argued for full relief from the outset.[50] In this context, control of 'total pain' looked like "managing pain, nausea, anxiety, depression, fear of the unknown, and their concern for their families."[51] Part of managing the anxiety was treating the pain preventatively instead of reactively (rejecting the notion of

[49] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 7.
[50] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 7.
[51] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 7.

gradual dosage increase). To further investigate optimal usage and administration of narcotics and other strong pain medication, they proposed a research program focusing on doctor–nurse–pharmacist collaboration, asserting that "a team of people can provide better than one person alone."[52] This furthers their emphasis on collaboration for the benefit of the patient, as well as the importance of interdisciplinary and cross-practice communication. Although they acknowledged the use of narcotics was controversial within medical communities,[53] the Planning group argued that this technique would significantly help the mission to fully relieve pain, enable patients to feel less dependent on their nurses, and live fuller lives unencumbered by cyclical reminders of pain as medication dosages wore off. The fight for these pain management techniques was well worth the possible results in their eyes.

With the departure from the typical hospital protocols, open communication about prognosis and care plans was necessary to provide patients with full autonomy. Hospitals lacked this need for communication, as care customization was not the norm. Wald and her colleagues advocated for open communication with patients—a significant departure from the long-standing practice of withholding upsetting information.[54] Taking inspiration from Kubler-Ross's investigation into communication between patients and providers and the significant de-tensioning and mutual comfort these individuals got from this experience, the planning group argued that honest dialogue would prevent unspoken emotional burdens and

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[52] Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 2.

[53] Newspaper article "'A License to Kill' Physicians Explore Euthanasia Catch 22," March 19, 1978, New Haven Register, 1659 box 10, folder 11, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

[54] Abel, *Prelude to Hospice*, 40.; Elisabeth Kübler-Ross, *On Death and Dying* (Routledge, 1973), 18, https://doi.org/10.4324/9780203010495.; Ruth Faden and Tom Beauchamp, *A History and Theory of Informed Consent* (Oxford University Press, 1986).; inevitable hour 94

help align care with individual patient needs.[55] These honest discussions of prognoses lessened feelings of failure for physicians, and managed the fear of the unknown for the patients confronted with their imminent mortality.[56] The patient-provider relationship was categorized as "understanding, agreement, and desire," underscore the depth to which this relationship was valued and central to this idea of hospice care.[57] Their envisioned hospice sought to create a flexible, honest, pain-free model of care centered not on cure but on comfort and personhood, with communication being a central principle.

In addition to the emphasis on communication with the patient, there was also an emphasis on communication with the patient's family and broader context. Continuing the ideas presented in the conclusions of the interdisciplinary study, this document emphasized close engagement between providers, patients, and families, arguing that familiarity with and adaptation to a patient's "life style" was essential for preserving decision-making agency. Repeated emphasis on this patient-family-provider relationship, "Hospice is shared among patients, families, and staff, all cooperating in a caring task," captures their conviction that hospice was not simply an emerging discipline but a particular structure for and facilitation of caring relationships.[58] Furthermore, it was a structure involving not only the dying individual, but also that person's support system and loved ones. This would also lay the foundation for family bereavement services.

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[55] Kübler-Ross, *On Death and Dying*, 17.
[56] Kübler-Ross, *On Death and Dying*, 17.
[57] Manuscript, "Guidelines for Patient Selection, Home Care Program" by Patient Care and Professional Relations Task Forces, August 7,1972, 1659 box 6, folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[58] Manuscript, "Philosophy Statement" by Hospice Planning Group, April 7, 1971, 1659 box 7, folder 88, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

This emphasis on familial relationships also reveals the intended scope of care and integration into the healthcare system. In these early days of hospice, they acknowledged that it was unrealistic to create a program that could provide the high-level care and pain management that they were targeting while also providing the day-to-day care that was already being fulfilled by home care agencies in the area.[59] The group identified environment as vital to well-being—an area where hospital settings consistently fell short. Thus, this envisioned hospice allowed individuals to spend their last days at home by customizing care to meet the patient's individual needs. Bringing medical care to the home prioritized emotional comfort, while providing structure for managing physical comfort. While fostering family involvement in care, this had the additional side effect of lowering costs, since the cost of dying in hospital was the same as receiving life-saving treatment in hospital. Dying patients no longer interested in curative measures would not live out their last days in intensive care paying exorbitant rates for care they did not want or need and would be in the comfort of their own home.

The Planning Group was also faced with the decision of the religiosity of their organization. With one-third clergy representation in the group, religious language (especially references to spirit and higher purpose) permeated their writings. Persons familiar with modern hospice rhetoric may not be surprised by the blending of religious and medical vocabulary, but it stands out as a deliberate choice at this early stage. Following in Saunders' footsteps, The Planning Group declared themselves a non-denominational organization. St.

[59] Pamphlet "Resources For The Elderly in the Greater New Haven Area" by the Junior League of New Haven, Inc., May 1967, Whitney Library at the New Haven Museum, RA 997.5.C6 J85.; Manuscript "Development of Core Staff for Patient Care" by Hospice, Inc. Board, July 1, 1972- December 30, 1973, 1659 box 6, folder 70, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, p 7.

Christopher's was also non-denominational, allowing both staff and patients to "define religion in [their] own terms."[60] Wald, Wessel, and Goldenberg concluded that they would form their primary institutional belief on the group commitment to the "needs of the human beings faced with a short period of life."[61] This would allow and support the contribution of any individual with this shared commitment, regardless of religious inclination.

A final notable aspect of this document is the equal treatment of nursing and medicine. The first main paragraph references "the professional and scientific knowledge of nursing and medicine,"[62] listing nursing first. Throughout the long history of interaction between nurses and doctors, there has always been a hierarchy.[63] Equating these two disciplines, and especially putting nursing first in the pairing, was a radical move for these authors, even considering Wald's identity as a nurse and dean of the Yale School of Nursing. This subtle disruption of medical hierarchy furthers the collaborative model of care, embodying the 'interdisciplinary' aspect of their envisioned hospice.

These discussions of central principles would become the foundation for the creation of a patient selection criteria for the Home Care Program, the staff hiring process, and the culture of the organization going forward.

The Home Care Program: Logistics and Launch

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[60] Minutes of Meeting, October 20, 1970, Box 7 folder 89, MS 1659 Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, 1.
[61] Minutes of Meeting, October 20, 1970, Box 7 folder 89, MS 1659 Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library, 2.
[62] Manuscript, "Philosophy Statement" by Hospice Planning Group, April 7, 1971, 1659 box 7, folder 88, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[63] Susan Reverby, *Ordered to Care: The Dilemma of American Nursing, 1850-194,* (Cambridge; New York: Cambridge University Press, 1987), 96.

By August 1972, the investigative work was complete and the intricacies of planning the Home Care Program were well under way. The Hospice Patient Care and Professional Relations Task Force was created to define the initial patient demographic, noting that the patient population would expand as the program itself grew.[64] Despite the idealism within their model, the document reflects careful attention to what services they could realistically provide, stating that these guidelines were intended to "optimize our capacity to deliver effective care to patients and families."[65] Visible in the numerous drafts of a document titled "Guidelines for Patient Selection," this task force debated logistics about their patient population including: maximum lifespan, requirement for cancer diagnosis, patient age minimum, patient's ability to pay, and their living circumstances.[66] These were all criteria worth debate, as the type of patient included in the program would vastly impact the resources and staff needed. The concluding hospice guidelines required that patients have a limited life expectancy (though no specific timeframe was provided), that patients be adults with a certain "maturity" (i.e. no children), that patients already have home-care services, and that they live within the greater New Haven area.[67] No conclusion on diagnosis requirement was made at this point.

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[64] Manuscript, "Guidelines for Patient Selection, Home Care Program" by Patient Care and Professional Relations Task Forces, August 7,1972, 1659 box 6, folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[65] Manuscript, "Guidelines for Patient Selection, Home Care Program" by Patient Care and Professional Relations Task Forces, August 7,1972, 1659 box 6, folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[66] Manuscript, "Guidelines for Patient Selection, Home Care Program" by Patient Care and Professional Relations Task Forces, August 7,1972, 1659 box 6, folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[67] Manuscript, "Guidelines for Patient Selection, Home Care Program" by Patient Care and Professional Relations Task Forces, August 7, 1972, 1659 box 6, folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

A patient's life span must be limited to qualify for Hospice Home Care. The document notably lacks an explicit cutoff or maximum for life expectancy, thus requiring a patient's acceptance to be on a case-by-case basis taking multiple criteria into account. This lack of qualitative standards more closely aligns with their general holistic and individualistic approach to patient care, though it would later have to change to fit into the bureaucracy of the American healthcare system. It was with the creation of the Medicare Hospice Benefit that a quantitative life expectancy threshold would be enforced.[68] However, this initial qualitative standard set the institutional tone.

Given this program's focus on patient autonomy and collaboration with physician and staff, they also required a level of maturity in their prospective patients. To give the patients the ability to make their own informed decisions about their care and goals, the patient needed to be able to make these decisions, thus excluding children from their initial patient population.[69] Despite the possible improvements that could have been made to care of pediatric terminal patients, the planning group prioritized patient autonomy at this stage of development.

In the Home Care program, care would be rendered in the patient's home, making living circumstances a critical aspect of the patient demographics. Though they described a variety of acceptable home situations (including hotels, personal residence, group living, and assisted living), enrollment in a home care program was required as Hospice would not be providing the full scope of care needed by these patients. This point in the criteria shows that

[68] MHB was created in 1982, implemented in 1983, and made permanent in 1986.
[69] Discussions of advance directives, while not included in this document, were undoubtably part of the conversation. This discourse began in the 1960s with attorney Luis Kutner created the first living will. This idea continued to grow in prevalence, though the Patients Self-Determination Act was not passed until 1990.

this hospice program was not designed to fully take over the care of the patient, rather assisting the structures of caring that already exist for this individual. It was an assistance program, not a replacement.[70]

Ultimately there was no clause about diagnosis (cancer or otherwise) due to task force fears about becoming known as a cancer hospital or a program exclusively for cancer patients. This was raised in a task force meeting in May of 1972 with the question, "is Hospice facility for terminally ill patients or is Hospice a facility for cancer patients?"[71] Though the clause requiring a cancer diagnosis was included in many previous drafts, including an initial patient population proposal from 1971 entitled, "Proposal for Planning and Development of a Hospice for Patients with Advanced Cancer,"[72] and a letter from potential physician Sylvia Lack in which the first point in the, "Current Criteria for Acceptance of a Patient/Family to the Hospice Home Care Program," was, "1. The diagnosis must be cancer,"[73] the clause was excluded for the founding of the home care program. This struggle with the identity of hospice as cancer hospital or terminal illness hospital would continue until funding from the National Cancer Institute limited Hospice's demographics to cancer patients in 1974.[74]

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[70] With one exception in which they describe a certain situation in which the family could use a break, and the hospice program could provide home care for a limited time. This is an early iteration of what is now known as respite care.
[71] Manuscript, "Minutes" for "Patient Care Task Force" on May 30, 1972, 1659 box 9, folder 113, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[72] Manuscript, "Summary Statement Proposal for Planning and Development of a Hospice for Patients with Advanced Cancer", January 11, 1971, 1659 box 7, folder 88, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[73] Sylvia Lack to Bob Canny (Chairman of Staff Selection Committee), May 1974, box 6, folder 71, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[74] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Once the logistics had been ironed out, the model of care was advertised and put into action. The early draft of their document distributed to drum up public support went through several title changes, each reflecting the group's evolving attempt to convey its mission. The original title for the document was, "To Honor All of Life: A National Demonstration Center to Protect the Rights of the Terminally Ill," which then became, "In Honor of Life's Ultimate Achievement: A Regional Center of Care for the Terminally Ill and Their Families," which then became, "In Honor of Life's Ultimate Achievement: A National Center for Teaching and Research."[75] All of these titles included the subheading, "The Case for Support of Hospice, Inc.," maintaining its purpose as a fundraising and public-relations tool.[76] Despite the editing and stark variation between iterations, this document was clearly made with the intent to gather public support for the hospice movement.

Underneath the subheading is a scratched-out quote from Hospice, Inc., President, Reverend Edward Dobihal, Jr.: "The extent to which we can humanize the process of dying – by consecrating death as a part of life – is a measure of our ability to humanize the process of living." This quote is replaced by Florence Wald's careful geometric cursive: "To help people live until they die," a clear attempt at streamlining the message portrayed on this title page. It is a difficult feat to convince the public of the merits of a new kind of care, in a new structure and hierarchy independent from any known and trusted hospital. This task becomes further complicated when the issue of death is involved. End of life is a fragile and emotionally difficult time and convincing the patient and family to trust a new model of care,

[75] Manuscript, "The Case for Support of Hospice, Inc.," Box 3, folder 25, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[76] Manuscript, "The Case for Support of Hospice, Inc.," Box 3, folder 25, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

unheard of in the United States and very new across the pond, was a precarious task needing careful word choice. Wald and her colleagues were aware of this fact, in that the drafts of this document for public release were marked up completely. In fact, there was not an untouched version of the document to be found in the Wald archive.

Conclusion

The influence of St. Christopher's Hospice is abundantly clear within the documentation of the planning that went into Hospice, Inc. The holistic approach to the individual in terms of family involvement, lifestyle consideration, and personalized expectations surrounding curative measures, all mirror the modern hospice foundations created by Cicely Saunders.[77] The adoption of Cicely Saunders' concept of 'total pain' and her pain management techniques is a direct connection between the two models of care. There are echoes of the philosophy of religion between the two hospices, with avidly religious founding members but non-denominational and religiously flexible organizations. While there are many clear instances of inspiration between the two, there are also clear departures from the blueprint established by St. Christopher's. The most striking of these differences is that fact that St. Christopher's opened as an inpatient facility that provided care under the English National Health Service,[78] while Hospice Inc., started as a home care service relying on private insurance payments and philanthropic grants. Though St. Christopher's soon expanded to include a home care program, and Hospice, Inc., inversely opened an inpatient facility in 1978, there are stark difference in their origins, both philosophical and practical. Hospice, Inc., started as an alternative option to dying in an

[77] Box 17, folders 92-95, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[78] Clark, *Cicely Saunders: A Life and Legacy*, 197.

impersonal institution with inadequate emotional support, later adopted to provide a more tailored facility for those needing higher levels of care in an inpatient setting. St. Christopher's started as an expansion of the church concept of a hospice, a place where terminal patients could forgo taxing curative treatment and receive deeper emotional and supportive care to ensure their comfort in their remaining time.

The journey towards a functioning hospice institution was far from over, but well on its way. A study had been conducted to determine the precise need of Americans with terminal illness. Experiential knowledge and theory had been collected from the mother of modern hospice, Cicely Saunders, in the form of lecture, written correspondence, and in-person discussion. An American hospice model of care had been conceived, a conglomerate of the wisdom from Saunders, personal experience from Planning Group members, research conducted in the community, and dreams of what hospice could be for the residents of the greater New Haven area. It was time to implement this work, begin gathering patients, and put this new American model to work.

Chapter Two

A Place of Their Own: The Inpatient Facility and the Making of a Local Institution

In March 1974, Hospice, Inc., successfully launched their Home Care program from their offices in New Haven, Connecticut. They were providing care to 165 patients in the Greater New Haven area by 1975, mostly sourced from Yale-New Haven Hospital oncologists. They were providing services such as wound care, bathing, and feeding assistance, and had enough demand and interest to necessitate a patient waiting list only a year after the program started providing care.[79] However, their work in establishing themselves as a legitimate healthcare provider was not finished. The goal to expand their services to include an inpatient facility still lingered, yet unfulfilled. New Haven Hospice had departed from the English model by opening the Home Care program first. St. Christopher's had opened as an inpatient unit, doing all the necessary fundraising and accreditation needed to create a facility providing that level of care. They had found their site, designed and built their facility, found staff, gotten accredited as a Nursing Home, and in that way found their patient pathway and success. Hospice, Inc., had done their fair share of support-gaining and fundraising, enough to get them a healthy Home Care program, but lacked the scale of support and funds they would need to found and build an in-patient facility from the ground up.

To expand their organization to this scale, Hospice, Inc., would need to extend their visibility in the public eye. Consequently, their reputation would also come to matter much more. This chapter explores the envisioned inpatient facility, Hospice, Inc., as it became more visible locally, their efforts to solidify their reputation, the major media discourse, and

[79] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

lastly, the opening of the inpatient hospice facility. I will utilize unpublished internal and self-published documents from the Wald archives to understand the motivation behind creating this inpatient unit, how it differed from the Home Care program, the argument for the necessity of the program, and the efforts to garner support. I argue that despite their belief that the Home Care Program was most important to the Hospice mission, they pursued the inpatient because it would expand the scope of care they were able to provide, but also because it would gain them establishment and respect. Care rendered exclusively in the home was not recognized by the medical community and the public any longer.

Goals of the Inpatient Facility

An inpatient facility was always included in the plans for Hospice, Inc., even from the beginning. St. Christopher's opened as an inpatient facility, directly modeled on the church hospices that were homes for the dying. In the church model, the patient demographic was largely impoverished people with no other home to go to.[80] With St. Christopher's as the primary inspiration for Hospice, Inc., the New Haven Hospice knew that they wanted to bring this type of care to their community in the United States, though did not see it as superior to home care. Having a physical location to render care would fully establish their institution as a modern health care center, making them competitive with hospitals and nursing homes. However, they knew that the amount of capital and support they would need to initiate was beyond their capability at that time. Thus, they founded the organization and put full effort into the Home Care program, a smaller and simpler project to undertake. Once the Home Care program reached a point of stability, Hospice, Inc., began working towards the daunting goal that was building, staffing, and running an inpatient hospice facility. Given

[80] Clark, *Cicely Saunders: A Life and Legacy*, 108.

the amount of attention garnered by the promise of an inpatient facility, it is easy to then postulate that Hospice founders felt the same. The inpatient facility was seen as the addition to the central Home Care program and not the other way around: "in New Haven we define the service as home care service with back-up in-patient beds."[81] However, as will be shown in this chapter, external perspectives from the public and media did not always reflect this hierarchy of programs. Though the inpatient wasn't the central component of the hospice program, the intensity of the efforts towards raising funds and support does show its importance. While not central, it was still critical.

Introducing this expansion of care required an expansion of the hospice philosophy. In the establishment of the Home Care program, heavy emphasis was placed on customizing care to the contours of each patient's life. In the inpatient model, the patients were displaced and thus their lifestyle impacted. These two concepts are contradictory, requiring a compromise. This compromise came in the form of an amendment to their intended patient population, this being the type of patient needing the level of care only available in hospital settings but not wanting to be in that setting. At that time there was no appropriate place for these patients to go, the options being either staying in the stressful hospital environment or moving home without adequate supportive care. In a panel discussion in 1975 with a group of nuns interested in starting their own hospice, Florence and Henry Wald contrast the concept of inpatient hospice with acute care facilities, visiting nurse association care, and nursing homes, as a "total system."[82] A disruption in symptom management, family

[81] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[82] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

involvement, or routine could be highly destructive for the very terminally ill. In the other existing systems, a step up or down in care would entail a completely new care team with no continuity of care plan, goals, or basic interpersonal familiarity. Consistent primary care physician, nurses, and social and spiritual support could be achieved with the integrated model of hospice home and inpatient care. Disruptions of any kind could be minimized to a high degree. While an acute care hospital could provide the continuity of care in terms of location and primary physician, "those acute hospitals have been built in order to treat and cure disease and we are talking about a group of people where those modalities are not successful which creates a lot of anxiety."[83] The missing emotional component was taken as a major flaw, given the centrality of emotion in the Hospice philosophy of care. Visiting nurse associations were discussed as important and helpful organizations, but ones that lack the physician management of the "delicate problem of symptom control," and encounters the same issues with continuity of care when a patients' needs change.[84] Nursing homes were deemed unable to cope with the medically complicated cases that Hospice was presented with, as these facilities were often understaffed and seen as places for geriatric patients to go and die.[85] Taking the drawbacks of the existing options into account, an inpatient facility was a necessity for the New Haven community. The envisioned inpatient facility was seen as a critical support for the Home Care program, specifically a place for the patients for whom acute care nor Home Care aligned with their needs and goals. Though this was considered a

[83] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[84] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[85] Memo from Hospice, Inc., "'Sidney Green. The motion passed," June 1974, Box 7, folder 75, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

necessary development, it was by no means replacing the Home Care program, which would remain the central program of Hospice.

Once this compromise was reached and the inpatient facility integrated into the hospice philosophy, the detailed planning could begin. The goals of the inpatient facility included providing "relief from distressing symptoms, the security of a caring environment, sustained expert care, and the assurance that they and their families will not be abandoned."[86] Though family members and carers were typically able to handle these symptoms in the home, there were instances when a patient progressed past their carer's ability and would benefit from more skilled, adaptive, and constant care. The question of creating an environment that could simulate the comfort of home and also facilitate all the levels of care needed was now raised. The unique features of the inpatient facility intended to accomplish this are outlined in the first Annual Report sent to the National Cancer Institute, as part of the funding agreement, in 1975. These features can be split into four main categories: physical facilitation of community, care, institutional structure, and finally, intentions for the planning process. Facilitation of community through the architecture and space was heavily emphasized: "space that allows family participation… and areas to permit children to be part of the community."[87] These features uphold a foundational principle of hospice philosophy: family participation in both care decisions and in the lives of terminal patients. A major part of foundational hospice philosophy was to integrate death back into everyday life. Thus, space for non-patients was a necessary inclusion in the plans. To include a variety of healing

[86] Article by Joan Craven and Florence Wald, "Hospice Care for Dying Patients," *American Journal of Nursing*, October 1975, box 20 folder 15, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[87] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

methods, care facilities beyond traditional medical equipment were integrated throughout: "a greenhouse corridor as transition space between patients' rooms and outdoor terraces, renewal and meditation spaces which provide secular and non-secular spiritual support… and appropriate space and equipment for recreation."[88] In these ways, the founders of Hospice continued their distinction from other existing systems of care by offering multiple modalities of support to patients, staff, and families. To continue to highlight the Home Care program, they included a clause suggesting the "architectural blending of services for home care and inpatient care to reinforce and make explicit coordination of these two options of care."[89] The Home Care program would be present and visible in this physical hospice location. Finally, to set intention for the rest of the planning process, they promised to "involve the community in the planning to develop strong and active ties between the Hospice and the population it serves."[90] This calls back to the importance of community needs and engagement in the development of Hospice. Hospice was not a carbon copy of St. Christopher's and was intentionally molded to the New Haven community. The range of features, from very concrete to purely theoretical, show the intensity of thought behind this addition to the New Haven Hospice. This envisioned facility had significant thought put into the ambience, with careful and intentional departures from typical hospital design, ingraining the Hospice philosophy into every brick.

In addition to the more philosophical choices, the medical capability of the inpatient unit was also closely considered. Though there would be no curative pursuit in the facility, some

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[88] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[89] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[90] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

medical equipment was still necessary. This included a fully stocked pharmacy with full-time pharmacist (primarily for pain management medications), diagnostic radiology, physiotherapy space, oxygen and suction systems for each bed, and a small laboratory for routine testing.[91] The choices of which equipment and systems to include were intentional to support comfort and were tailored to the patient demographics. They did not follow the construction of a typical hospital and instead chose to include only the necessary technology for their patients.

However, the necessity of the institution did not guarantee its existence. In the wake of the successful Home Care program, Hospice, Inc., shifted their efforts towards communicating this necessity with the intent of gathering support and funding. As Wald articulated in the same conversation from above: "you can dream your own world but it takes a lot of time and that construction money is very difficult to get."[92] As soon as the organization could allot the time and effort needed to pursue funding, they began. They managed to purchase a building site at the end of 1975.[93] However, they had not yet raised enough to begin the next step: construction. In a report distributed in 1975: "The problems of funding and fund raising continue to cause concern. In order to break ground for the planned inpatient facility in April 1976, Hospice must raise $1,000,000 beyond the funds already received."[94] They were ultimately unable to raise the funds to start construction at this initial expected time, and the groundbreaking ceremony was not until two years later in November

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[91] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[92] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[93] Hospice, Inc., Newsletter, January- March 1976, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[94] "Annual Report to the National Institute of Health" 1975, Hospice, Inc., box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

1977.[95] The timeline would continue to get pushed back not only by funding limitations, but also by bureaucracy as they pursued building permits, local town approval, state board approval, and hospital certification. Though they were ultimately successful, the external bureaucratic forces would largely prove to be the limiting factor in this process.

Efforts Towards Support and Legitimacy

In order to plan and construct a building, staff it with qualified employees, and fill it with patients, there were an abundance of bureaucratic hoops to jump through. Establishing the Home Care program was not nearly as complex, just requiring a business license and a staff of registered nurses and physicians. It was due to this bureaucracy that successfully creating an inpatient facility would establish Hospice, Inc., as a legitimate healthcare provider in a way that their successful Home Care program did not. To understand these external influences, I will expand my analysis to the materials that Hospice, Inc., published during this time of fundraising and approval-seeking to show how the organization was appealing to these local, state, and federal influences.

Hospice, Inc., made several strategic changes to better garner support and manage the media attention. These changes included appointing a Director of Public Information and Education by 1976, and it was this individual that provided information or organized interviews for publications. The creation of this role emphasizes the importance of the media in this next stage of Hospice development. Even earlier, Hospice began publishing a newsletter to spread the updates on their development and gather support and funds. Though their pursuit of larger grants for the planning and construction of inpatient didn't begin until

[95] Pamphlet "hospice Groundbreaking," November 20, 1977, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

1975, their solicitation of monetary support from their community began as early as 1973. In their February issue from that year, they share their "first solicitation" attempt of offering the "opportunity for task force members to purchase 'shares' in the Hospice site at $1.00 per sq. ft."[96] In this same newsletter, the organization informally requests any ideas for the "launching of a fund raising drive for purchase of the Hospice site!"[97] These early newsletters have the rhetoric of correspondence with a close friend as opposed to a mission statement or advertisement for their programs. There is little in terms of persuasive argument in the newsletters from 1973. However, the inklings of argumentative rhetoric begin to appear in the latter half of 1974, especially as the architectural plans finalized. In the December 1974 issue, there is a passage about the notable and attractive aspects of the plans, including the proximity to New Haven, the "home-like environment," the common areas filled with plants "suggestive of a greenhouse", the single-level structure supporting maximum freedom and flexibility for patients, the option for privacy for both patients and staff, and the 24/7 pharmacy.[98] This is the first example of blatant self-advertisement for the inpatient facility in these newsletters, showing a steady shift in the newsletter's purpose to Hospice, Inc.

[96] Hospice, Inc., Newsletter, February 1973, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[97] Hospice, Inc., Newsletter, February 1973, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[98] Hospice, Inc., Newsletter, December 1974, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Figure 1: Hospice, Inc., Newsletter Logo Progression.

From Hospice, Inc., newsletters, MS 1659, box 7, folder 86, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut. A progression of Hospice, Inc. newsletter logos from 1974 to 1976. "Hospice… a vision" to "News from Hospice" to "hospice." What begins as a clearly hand-drawn logo with the uncertainty of an ellipsis, gains confidence with the declaration of "news from," and finally announces itself with the polished gothic font logo debuting in 1976.

They continued to expand their persuasion in the January-March newsletter of 1976 with the first justification for the role of the inpatient facility: "the inpatient facility will provide a much-needed support system for those patients for whom Home Care is neither appropriate nor desirable."[99] The lack of justification in earlier newsletters points to the intended audience of the newsletter as supporters, not skeptics. However, the newsletters were far from their only tool for outreach. To convince skeptics and the broader community, members of Hospice, Inc., were busy with speaking engagements in the community, television programs, and creating brochures and pamphlet material for their growing programs. These other forms of outreach were more heavy-handed with the persuasion while the newsletters maintained their updates and non-confrontational tone.

This approach to publicity saw major results. In their 1975 annual report to the National Cancer Institute (a major source of funding for the Home Care program), they outlined the organization's success in community relations:

> We have responded to over 200 written requests for information about our Hospice program from residents or organizations in 36 of our fifty states. Our standard mailing list for general information, newsletter, etc., now totals more than 2,500 names. In order to better inform our readers, the style, format, and design of our newsletter was changed. Our work and programs have progressed to the point at which we are no longer 'a vision.' We are now a reality providing care to patients and their families.[100]

[99] Hospice, Inc., Newsletter, January- March 1976, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[100] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

This emphasis on their changes to their popular newsletter underscores their perceived importance of their reputation and the varied ways in which they are curating their interactions and image. In addition to the outpouring of interest from individuals across the state and nation, the media was covering the New Haven Hospice Developments with fervor.

Media Coverage and Funding: 1971-1977

To gain an understanding of New Haven Hospice's reputation during this time, I will be using the attention of the press. Although this does not directly communicate the reputation of Hospice, Inc., it does reveal the major points of contention that were raised throughout the organization's development, as well as the views of the people passionate enough to report themselves or get in touch with the press. Archives of the oldest continuously published newspaper in the country, The Hartford Courant, provides insight into the local media coverage of hospice throughout its early growth. This in combination with the extensive newspaper clippings that Florence Wald collected in her personal archives tells an extensive story of this period of hospice development. It is not a complete picture, but, as opposed to the internal documents spread by Hospice, Inc., it shows dialogue not curated by the administrators.

In this era of development before the groundbreaking of the inpatient facility in 1977, the media coverage ranged widely in style. However, some patterns emerge from this wide sampling. Notable issues covered by the media coverage during this time include, the emphasis of the home care versus inpatient program, discussion of the cost of care, comparisons to nursing homes, the patient demographics, the usage of the word "cancer" versus "terminally ill," the mention of bereavement services, public support, logistical

updates, education initiatives, pain management techniques, usage of patient narratives, and mentions of English hospice.

The headlines from these articles are telling of the attitudes towards death and the success of the hospice concept. Many of these earlier articles, especially the more logistical articles, refrain from using the words "death" or "dying" in their headlines. Any inclusion of these words is accompanied by a soothing word such as "dignity," or specifying that it is "care for the dying," or "to help dying".[101] The word "dying" is never related to human beings by saying 'dying patients' or 'dying people', and if any mention is made towards an individual they are referred to as "terminally ill," not dying.[102] The fact alone that almost every article from 1971 onwards refers to Hospice, Inc., as simply, "Hospice," suggests a widespread familiarity with the term, showing the speed with which the concept was beginning to spread.

Even in the location of the articles in their respective newspapers, we see a developing fame. Comparing two longer articles that go into depth of the Hospice philosophy, include interviews with organizing members, summarize professional and public reception, and provide logistical updates, from only two years apart, a shift is noticeable. The first, "Medical

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[101] "Proposed Facility To Help Dying Set Back," *Hartford Courant*, October 25, 1973, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/551895598/fulltextPDF/F4780B80542A4EECPQ/36?accountid=11311&sourcetype=Newspapers.; Linda Finhorn, "New Hospital Concept Seeks To Attach Dignity to Death," *Hartford Courant*, March 18, 1973, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/551468675/F4780B80542A4EECPQ/42?accountid=11311&sourcetype=Newspapers; "Medical Institution To Provide Care for The Dying," *Hartford Courant*, November 28, 1971, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/550973906/F4780B80542A4EECPQ/43?accountid=11311&sourcetype=Newspapers; B. D. Colen, "Death With Dignity - at Home," *The Washington Post*, November 16, 1975.

[102] "Terminal Cancer Home Gains $50,000 Grant," *Hartford Courant*, July 19, 1973, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/551694572/fulltextPDF/F4780B80542A4EECPQ/53?accountid=11311&sourcetype=Newspapers.; Newspaper article, "Terminally Ill Find Relief at Hospice," 1976, *The Yale Daily News,* box 11, folder 10, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Institution to Provide Care for the Dying" published in the Hartford Courant in 1971, appears on page 43A. Two years later, "New Hospital Concept Seeks to Attach Dignity to Death" also published in the Hartford Courant, appears centrally on the front page. This relocation happened early in the development timeline of the New Haven Hospice and shows the speed with which they gained public recognition and attention.

The contents of the articles further reveal public attitudes towards hospice, including the major points of interest, contention, and confusion. One major theme was the mention of the specifics of the hospice programs. In the vast majority of articles I analyzed, both the home care and inpatient programs were mentioned with equal significance, despite the existence of neither or solely the home care program at the time of publication. I attribute this to the familiarity of a healthcare program that occurs within a physical healthcare facility. Long past was the concept of a physician home visit the norm. While medical professionals coming into the home did occur during this time (i.e. Visiting Nurse Associations, Emergency Medical Services, etc.), this type of care was reserved for a select demographic and therefore would not have been familiar to the average reader. While I see no internal documentation acknowledging this fact, I do wonder if Hospice founders took this into account when considering the timeline of their development. Regardless, by the frequency of mention, it is clear that having the inpatient facility formed Hospice into a more recognizable and respectable institution in the eyes of the public.

While the bulk of words used in reporting on Hospice were used in the longer articles communicating philosophy and origins, the logistical articles were by far the most frequent publication. These logistics included funding received, announcements of open houses and events, and bureaucratic updates. With titles like, "Hospice Agent Will Speak About Cancer,"

or, "Hospice Facility Given $20,000," or, "Hospice Opens Fund Drive for 44-Bed Facility," and article body of often fewer than 100 words, these articles were brief, direct, and almost without emotion.[103] Conceivably, this reflects the intended audience of these articles, they were not to drum up criticism or support, but merely a way to keep interested individuals in the know. Dramatic language was not necessary to catch the attention of these individuals. The inclusion of these logistical updates in a state-wide newspaper at all, and on a regular basis, reveals the importance of the growth of Hospice, Inc., to the population of Connecticut.

Several more contentious issues came up in the press coverage, including comparisons to nursing homes and the use of pain medication. During this period, nursing homes had peaked in numbers and profit, but had also gained many critics. Often viewed as understaffed, money-grabbing "deathhouses," a comparison to one was not a compliment. Describing Hospice, specifically the inpatient facility, as a, "place to die," was a common quotation which the reporter sometimes defended, or other times included without commentary. One particularly feisty article makes their claim known in the headline: "Hospice Is Called No Nursing Home," in which Dobihal is quoted to describe that firstly, the patient ages range from "19 to 89 with an average of 50," differing greatly from the geriatric nursing home population and secondly that, "about half of [our patients] have died

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[103] "Hospice Agent Will Speak About Cancer: Thomaston," *Hartford Courant*, February 16, 1977, ProQuest Historical Newspapers, https://www.proquest.com/hnphartfordcourant/docview/544860849/fulltextPDF/49DF4B3E98754FE7PQ/4?accountid=11311&sourcetype=Newspapers; "Hospice Facility Given $20,000," *Hartford Courant*, January 13, 1976, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/544399083/F4780B80542A4EECPQ/25?accountid=11311&sourcetype=Newspapers; "Hospice Opens Fund Drive for 44-Bed Facility," *Hartford Courant*, December 17, 1974, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/pagepdf/552436085/Record/F4780B80542A4EECPQ/30?accountid=11311&sourcetype=Newspapers.

at home," and not in facility as would happen in a nursing home.[104] Though this defense does not explain the difference between nursing home and *inpatient* hospice care (i.e. care for those not dying at home), other articles do. One such article defends inpatient hospice by describing the opportunities granted to patients in their care: "to not be hooked up to every possible kind of device, to be given comfort, to be given his mind back, to receive friends, to be able to sit up because the pain has been controlled," implying that the same would not be true for a patient in nursing home.[105] Another differentiates the care on the basis of rehabilitation: nursing homes pursued return to activity, while hospice emphasizing comfort.[106] The mention of pain medication usage is subtle, and no overt critique was found in any of the analyzed articles, though some mention other physicians' hesitancy to use so-called dangerous and addictive drugs. It is only in articles not specific to the New Haven Hospice that use of addictive pain medication for the treatment of cancer and terminal illness is criticized.[107]

Within these contentious conversations, Hospice, Inc., kept a close eye on the media coverage, specifically Florence Wald. Consistent with Wald's tendency to micromanage[108], her attention to the press was laser-focused and she was quick to defend and correct the articles published about Hospice, Inc. Her meticulous collection and categorization of articles

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[104] "Hospice Is Called No Nursing Home," *Hartford Courant*, April 7, 1976, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/544523153/F4780B80542A4EECPQ/21?accountid=11311&sourcetype=Newspapers.
[105] *Hartford Courant*, "Medical Institution To Provide Care for The Dying."
[106] Finhorn, "New Hospital Concept Seeks To Attach Dignity to Death."
[107] Newspaper article, Harold Schmeck, Jr,"Addictive Drugs as a Way of Easing Death," *New York* Times, November 1977, box 11, folder 10, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library. Newspaper article, "Rocky's Do-It-Yourself Death," *New Solidarity,* box 11, folder 10, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[108] Abel, *Prelude to Hospice*, 15.

published about any kind of hospice shows the seriousness with which she treated media attention and her protective nature. This was an important aspect of feedback that the organization was receiving from the public, one worthy of close attention. In one folder of her clippings, there is a letter from Wald to Jean Krow, the editor of *New York Magazine* in 1976. In this letter, Wald thanks Krow for the "sensitive and well-written article" about Hospice, Inc., and then goes on to correct several statements made in said article. Wald corrects the language used to describe the confrontation and centrality of death within the Hospice program, and goes on to defend the treatment of dead bodies in large hospitals saying "I don't believe that health workers regard the human body as crassly as the quotation suggests," and suggests that it is the size of those institutions that discourage depersonalization.[109] She closes the letter by requesting its publication in the following edition of New York Magazine so that readers could, "correct these two issues in their mind."[110] The attention to detail and careful consideration of every word choice seen in the drafting of internal documents extended to the words chosen by others, revealing the criticality of public perception of hospice during this time, and the lengths to which Wald and her colleagues would go to ensure a positive portrayal.

Ongoing during these philosophical and opinionated discussions are successes and failures in the world of fundraising and site acquisition. Hospice was successful in being given several significant grants during this time, during and after the foundation of the Home Care program. On a small scale, this success came in the form of significant local support

[109] Florence Wald to Jean Krow, 2 March 1976, MS 1659, box 11, folder 10, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.
[110] Florence Wald to Jean Krow, 2 March 1976, MS 1659, box 11, folder 10, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

including multiple local fundraising events bringing in an excess of $5,000 each.[111] In addition to this strong showing of local support, Hospice received several large institutional grants. This did not completely alleviate the stress that fundraising on this scale entails, but these two sources of support made great progress towards their goal of $2.4 million (which later grew to $2.7 million in 1976, and ultimately $3.5 million).[112]

However, attempts to acquire a building site and certification, which began in 1973, were not initially successful. In 1973, Hospice acquired approval to build, provided they find and begin the process of purchasing a site within three months. They were unable to meet this criterion, and their approval expired in October. This early failure was primarily a result of lack of reputation and funds, contributing to their later dedication towards establishing themselves on these fronts. Their initial attempt at finding a site was focused on the towns New Haven, Ansonia, and Seymour, as dictated by the State Office of Comprehensive Health Planning, because these towns were experiencing a shortage of nursing home beds.[113] This equation to nursing home status was not helpful in acquiring funds or a building site due to nursing homes' lack of popularity, and they were unable to drum up enough support (fiscal and political).[114] In the wake of this rejection, the timeline for the construction of the facility was shifted from an expected opening date in 1975 to 1977, and efforts towards building their state-wide reputation and capital were intensified.[115] Only 8 months later, they again

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[111] Hospice, Inc., Newsletter, January- March 1976, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[112] "Inpatient Facility Summary Analysis and Recommendations: A Report to the Board of Directors of Hospice, Inc., September 1975"; Hospice, Inc., Newsletter, February 1976, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.; Newspaper article, "Funding Out for Hospice," *Branford Review*, January 20, 1977, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[113] *Hartford Courant*, "Proposed Facility To Help Dying Set Back."
[114] *Hartford Courant*, "Proposed Facility To Help Dying Set Back."
[115] Finhorn, "New Hospital Concept Seeks To Attach Dignity to Death."

received approval (with the same conditions), and were able to purchase a site by the end of 1975. On this second attempt, there were two significant changes. First, they shifted their geographic focus from the towns recommended by the state office and instead pursued the town of Branford. This town was appealing due to its proximity to downtown New Haven, flexible zoning laws, and local support. Second, by 1975, Hospice, Inc., had received significant enough financial support to be able to afford purchasing a site.

The first substantial grant Hospice, Inc., received towards the inpatient facility was from The Commonwealth Fund in 1973. The Commonwealth Fund, founded in 1918 and still in operation today, focuses on supporting independent research on health care issues and improving health care practice and policy. In the 1970s, the Commonwealth fund was supporting a variety of causes, including funding reform of medical school curriculum. The choice to support the Hospice movement was a significant choice on the part of the fund, proving both that Hospice worthy of support and a necessary development in medicine. In an article describing their support of changing medical school curriculum, the fund's president describes widespread dissatisfaction with medical education saying, "virtually no one in the university world seems to be content with the pre-medical and pre-clinical sequence… but that change is difficult or impossible. We believe it is possible and we are challenging the universities."[116] The circumstance surrounding medical school curriculum has remarkable similarities to the discussion surrounding death, in which many healthcare workers express dissatisfaction with the dialogue around and treatment of the dying, but were hesitant to introduce change. In the realm of care for terminal patients too, the Commonwealth Fund

[116] Clipping of article "Doctors' Training Subject of Study" by Lawrence Altman in *The New York Times*, 23 November 1975, MS 1659, box 11, folder 10, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

supported the cause to change the norm despite the hesitance. This early show up support was important for the morale in the fundraising effort, as well as encouragement for other organizations and individuals to invest. This grant, providing funds over a period of two years, was given to be used for the planning of the Home Care program, as well as the future building.[117]

Also in 1973, Hospice received a grant from the Ittleson Family and the Van Amerigen Foundation towards the planning of the blossoming Hospice.[118] Though these two grants were specifically for the home care program, and not the inpatient facility, they served to help the inpatient mission by removing the stress of finding funding for both programs. With Home Care covered, all effort could be put towards continuing to raise funds specifically for the planning and construction of the inpatient facility. This would be fruitful, as by early 1975, Hospice had managed to secure a sizable grant from the Kellog foundation and from the National Cancer Institute, reaching half of their funding goal.[119] Though this goal was not the entire $3.5 million, it was a feat that they managed to reach this major milestone in only two years. These grants in combination with the local funds, philanthropic and governmental, secured the organization's ability to pay the architect and continue forth on the journey towards construction.

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[117] Linda Einhorn, "Hospital Group Gets $200,000 in Grants," *Hartford Courant*, March 25, 1973, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/551446547/fulltextPDF/F4780B80542A4EECPQ/37?accountid=11311&sourcetype=Newspapers.

[118] Hospice, Inc., Newsletter, February 1973, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

[119] Manuscript, "Inpatient Facility Summary Analysis and Recommendations: A Report to The Board of Directors Hospice, Inc.," box 7, folder 75, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.; "Hospice Raises Half of Goal," *Hartford Courant*, December 9, 1975, ProQuest Historical Newspapers: Hartford Courant, https://www.proquest.com/hnphartfordcourant/docview/544225600/F4780B80542A4EECPQ/23?accountid=11311&sourcetype=Newspapers.

Reaching the Inpatient Finish Line

Despite the broad success Hospice, Inc., had with funding, the construction journey did not continue without hiccups. Hospice did not describe these failures in such harsh tones in their own newsletter, but media coverage did not mince their words. A local Branford paper covered the organization's failure to obtain enough funding in 1977 to continue the construction timeline as planned.[120] The article describes the federal fund that Hospice applied to, and the news that they did not receive funds in that round. These funds were needed for the planned groundbreaking ceremony, and after the disappointment of not receiving the funds, the groundbreaking ceremony was delayed from January 1977 to June and ultimately to November. The local media coverage carefully shields any emotional response to this setback, though it peeks through in the coverage of the local pushback against the decision to deny Hospice funding, and in the encouraging subheading of this article: "Maybe next time."[121]

After the groundbreaking ceremony in November 1977, the inpatient facility was completed in two years, opening in October of 1979. To welcome in a new era of Hospice care, the organization changed their name from Hospice, Inc., to Connecticut Hospice, Inc., or more commonly, Connecticut Hospice. By this point, their newsletter had 12,600 subscribers, and copious media attention and interest.[122] Their increased scope, funding, and

[120] Clipping of article, "Funding Out for Hospice," *Branford Review,* January 20, 1977, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[121] Clipping of article, "Funding Out for Hospice," *Branford Review,* January 20, 1977, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[122] Booklet, "Hospice 1978 Annual Report," box 7, folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

establishment allowed them to treat an average of 700 patients per year, beckoning in a new era of Hospice in Connecticut.[123]

[123] Booklet, "Hospice 1978 Annual Report," box 7, folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Chapter Three
Teaching Hospice: Outreach, Education, and the Consolidation of Institutional Authority

From the outset, the founders of Hospice Inc., hoped to spread the hospice philosophy and intended to serve as a role model for new hospice care organizations. As will become clear in the following chapter, explicit education for the purpose of establishing more hospices was not always their first priority, but sustained efforts to spread hospice philosophy were made throughout the development of Hospice, with reciprocal interest from communities across the United States. This chapter will discuss the development of their efforts to inform, moving from their internal staff training programs to local speaking engagements, workshops, and finally creating and implanting curriculum in other schools. Given the scope of this project, I will not be discussing every development or example of their education initiatives but instead will approach the progression with a collection of case studies given the materials available in the archive. I argue that this progression of outreach and speaking engagements reflects the emerging authority of Connecticut Hospice on the issue of care for the terminally ill and the dying during this period from 1972 to 1978. To gain this authority, Connecticut Hospice leveraged their experience with cancer to make the case for their unique expertise in terminal care.

Educating from the Inside Out: Staff Training

The first step in beginning to spread the knowledge and philosophy of hospice was to create a strong internal education program for the staff working at Hospice. As their facilities grew from a planning group to a Home Care program providing services to patients, Hospice needed provider staff beyond the physicians and nurses in the original planning group. After

finding fitting candidates, training these staff in-house was the critical to launching the Hospice facility and its future, as the principles of hospice were not taught anywhere else at this time. Training these new employees would also be the first step towards spreading the ideals of Hospice with the nation, as inevitably some employees would move on to other jobs and employers and could carry on the hospice philosophies, in this way beginning to pollinate the healthcare field with hospice approaches. To teach the staff, the planning group developed a specific six-week course, entitled, "The Interdisciplinary Experience in Care of the Terminally Ill and Bereaved for Health Care Specialists," originally called the "Multidisciplinary Health Care Team Education Curriculum." Hospice Inc., organized the course with support from Cicely Saunders and Elizabeth Kübler-Ross to train and prepare the staff of Hospice on the principles of caring for terminal patients in this new system of care. The course's curriculum exposed staff to the emotional difficulties associated with death and dying, provided education on approaching these difficult issues, and introduced general institutional protocols.[124] Though only used internally, the depth of the curriculum shows the expertise with which the foundational group was able to teach the model they had created.

This internal curriculum focused on cancer as the primary affliction of their patient population. The connection between hospice care, terminal illness, and cancer was tenuous from the start, highlighted by the debate of Hospice's identity as a cancer hospital and the conversation around requiring a cancer diagnosis or not.[125] But even before the foundation of Hospice, Inc., the connection was there. In the time before effective curative cancer

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[124] Manuscript, "Curriculum Outline: Interdisciplinary Experience in Care for the Terminally Ill and Bereaved for Health Care Specialists," Hospice, Inc., 1976, box 7, folder 77, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

[125] Manuscript, "Guidelines for Patient Selection, Home Care Program" by Patient Care and Professional Relations Task Forces, August 7,1972, 1659 box 6, folder 73, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

treatments, cancer as a descriptor was interchangeable with terminal illness, with doctors pronouncing a patient as, "incurable months and sometimes years before they died."[126] Many of the studies of the terminally ill and their needs, including Kübler Ross's and Wald's, both were conducted in oncology wards with an almost exclusive demographic of cancer patients.[127] Furthermore, the very idea of easing an individual into death as painlessly as possible has the implicit requirement of a slow and predictable decline to death. Many causes of death do not follow this course, as many deaths are sudden and difficult to predict (e.g. heart attacks, fatal injuries, stroke, aortic aneurysm), chronic conditions that do not have predictable progression (e.g. Chronic Obstructive Pulmonary Disorder, dementia), or ill-defined diagnoses (e.g. failure to thrive). Cancer, though of course not in every case, is often a progressive decline that can be anticipated once curative measures are ineffective. This makes it a difficult disease to experience, but also a disease that is well-suited to the type of personalized, progressive care that hospice provides. Though there is an incredible diversity of cancers, many rare, when grouped together there is a significant patient population: approximately 385,000 cancer deaths in the US in 1977 (~1.75 per 1,000 people), and approximately 615,000 cancer deaths in 2025 (~1.79 per 1,000 people).[128] For these reasons, cancer patients have been and continue to be a central demographic in hospice care. This, in combination with the "incurable" association it gained in the early twentieth century, explain the cancer focus in their internal curriculum on terminal illness. The question of centering cancer patients as the primary demographic was hotly debated throughout the foundational

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[126] Abel, *The Inevitable Hour*, 6.
[127] Kübler-Ross, *On Death and Dying*, 2.
[128] Table "Estimated Cancer Deaths, Services Needed, and Nursing Work Force Required in the Fifty States and Puerto Rico," 1977 ,American Cancer Society ,box 20, folder 2, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.; American Cancer Society, *Cancer Facts & Figures 2025*, 2025.

years of Hospice, Inc., but was ultimately decided by the grant from the National Cancer Institute in 1974.[129] This grant fully funded the new Home Care program with the stipulation that the program would care for exclusively cancer patients.[130] Though this debate would resume once the NCI grant ended, this important institutional identity was decided for them and is reflected in this internal curriculum.

Given their federal funding limiting hospice care exclusively for oncology patients, the staff training course focused on cancer. Accordingly, the course curriculum addressed potential questions staff might have about cancer in a section called, "Answering Your Questions About Cancer."[131] Within this section, the emotional components of the disease are covered first in a unit called, "Feelings are Facts." Consistent with the emphasis of the holistic approach to care in modern hospice ideology, the emotional components are given central stage, with logistical information put secondary. This placement not only reflects the hospice philosophy but also tells the student to give emotional symptoms priority. The program continues to medical details of cancer with, "Several Hundred Different Diseases", "Immunologic Aspects of Etiology", "Principles of Management – Cure, Palliation" with following subunits about surgery, radiation therapy, chemotherapy, endocrine therapy, and immunotherapy. These sections emphasize the diversity of causes and experiences of cancer on the pathological level, showing the ability of the curriculum writers to communicate hard facts as well as more theoretical philosophical core principles. The notation distinguishing 'cure' and 'palliation' is the very core of the hospice idea, and this distinction emphasizes the

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[129] See chapter 1 for more on this

[130] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

[131] Manuscript, "Curriculum Outline: Interdisciplinary Experience in Care for the Terminally Ill and Bereaved for Health Care Specialists," Hospice, Inc., 1976, box 7, folder 77, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

perceived importance of teaching this difference, as most likely the new staff would not be knowledgeable of this idea.

The focus remains on the care of terminally ill cancer patients, but the second part of the curriculum mainly concerns the role of the provider in caring for the cancer patient. The majority of their new staff were registered nurses and licensed practical nurses, though it is unclear if any of the physicians hired for the inpatient program participated in this course. This section gives the practitioner three distinct identities: "a skilled practitioner," "a coordinator and utilizer of community resources," and "a change agent." These are surprisingly abstract terms to describe the role of a health care worker, but the expansive expectations laid on the employees of Hospice, Inc., shows the idealistic and unrealistic aspects of early hospice philosophy. Furthermore, this abstraction of the typical role of the provider includes utilizing local resources and puts the authority of care on the practitioner by giving them the authority to contribute to reforming end-of-life care. In this way, this curriculum communicates not only the necessary logistics of providing care, but the philosophy underlying the hospice model of care.

As reflected in the curriculum, the emotional symptoms and experiences of the patient were to become a focus of hospice care. Though there is immense detail spent on the specifics of different cancer progressions and symptoms, including prostheses, mobility exercises, tracheostomy care, and altered elimination (incontinence), to name a few, more than half the curriculum units discuss the emotional aspects of dying from cancer, and the corresponding actions to be taken in care to manage them, for both patient and practitioner.[132]

[132] Manuscript, "Curriculum Outline: Interdisciplinary Experience in Care for the Terminally Ill and Bereaved for Health Care Specialists," Hospice, Inc., 1976, box 7, folder 77, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Including a range of specificity, from the general, "Psychological Distress in Terminal Illness" to, "Psychologic Adaptation to Ostomy," the patients' emotional state is considered and acknowledged at every level and step of this curriculum, once again reflecting the holistic hospice approach. Integrating the logistical and emotional information was an intentional choice, both for raising awareness of the emotional involvement but also encouraging the staff to consider the emotional state of the patient and integrate this consideration into their care. In recognition of the emotional toll of noticing and responding to patients' emotional state, the curriculum includes processing time for trainees. This is notably in stark contrast to the working environment at St. Christopher's in England, where the culture was described as, "authoritarian and inflexible, concerned only for the patients and not for the staff."[133] Letting the trainee process any emotions that arise when confronted with the details of caring for cancer patients is emphasized: "depression may be experienced by the trainee… it is importance for him to be in a supportive environment at this time so that he can be helped to learn ways to cope with his own feelings."[134] Though I saw no evidence of this curriculum being shared with other institutions, the extensive justification and commentary on methodology raises the question if this was written with the intention of sharing. The six-week length of the program is justified, both so that the trainee can see the trajectory of a patient in their entirety, or what they described as the full journey, "from illness through bereavement."[135] This generous length also provided time for adjustment to

[133] Clark, *Cicely Saunders – Founder of the Hospice Movement*, 130.
[134] Manuscript, "Curriculum Outline: Interdisciplinary Experience in Care for the Terminally Ill and Bereaved for Health Care Specialists," Hospice, Inc., 1976, box 7, folder 77, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[135] Manuscript, "Curriculum Outline: Interdisciplinary Experience in Care for the Terminally Ill and Bereaved for Health Care Specialists," Hospice, Inc., 1976, box 7, folder 77, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

the new and emotionally weighty work environment. Continuing the intentional construction of this course, each trainee was provided a preceptor to act as a guide alongside the workshops, clinical experiences, lectures, slide presentations, discussions, and conference attendances. This started a culture of generational and experiential knowledge and brought the concept of community support and engagement into the internal structure of the institution.

After more than five years of institutional knowledge synthesis and establishment, the creation of this curriculum was a milestone the beginning of the documentation of institutional knowledge, a key preparatory step for the larger education initiatives to come, including workshops and conferences. The detailed outline of schedule, content, and teaching methodology reveals the carefulness with which the organization approached this preparation. The document outlining their curriculum had "draft 2" scrawled at the top of the page, indicating several iterations of calculated curation. Florence Wald's experience as Dean of the Yale Nursing School and prior curriculum-making experience supported the organization's ability to make an effective internal course. She was a key contributor to the Yale Nursing School expansion from a baccalaureate to a master's degree program in 1959, allowing students to have more specialized focus within the broad field of nursing and supporting the shift towards nursing science. She was passionate about adapting to the changing demands on nurses, and her work as dean reflected this.[136] Wald's expertise resulted in a program providing new hospice staff with a physical and psychosocial understanding of

[136] Radio conversation between Bernard Mullins and Florence Schorske Wald, "Yale Reports Number One Hundred Forty One – The Function of Nursing in Modern Society – The Changing Role of the Nurse," February 15, 1959, box 19 folder 9, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

cancer, clarity of their own emotions surrounding death and dying, the ability to recognize covert needs of patient and family, plan and implement a care plan, and facilitate the involvement of other care providers and available community resources. Thus, the philosophy was put into practice.

Publicizing Hospice Through Speaking Engagements

Developing simultaneously with the internal curriculum, members of the planning group began speaking about Hospice. The early engagements were primarily awareness-raising, though the tone would develop into self-advertisement as time went on and their authority grew. Consistent with Wald's archival tendencies, there is ample evidence of Wald's speaking engagements over the years.[137] Wald spoke from her experience as a Dean of the Yale Nursing School, an experienced nurse, a foundational member of the Hospice, Inc., Planning Group, and a curriculum creator throughout the development of hospice. I differentiated the intent between the various speaking engagements (personal versus institutional) immortalized in the archive through the content of the talk (often from the title of the talk, if a summary or transcript is not available), the specific request and language used, and the other speakers in the case of a conference or panel (i.e. did other Hospice organizers speak).

One of the earliest hospice speaking engagements was an invitation to Wald from the National Student Nurses' Association for their 22nd Annual Convention in 1973. Just the previous year, the convention committee had added a panel on aging. This panel had been met with great interest from conference attendees, and the organizers invited Florence Wald

[137] Though I am not focusing on Wald's engagements alone, I have the most information on her and her husbands, given the content of the archive.

to contribute to the panel the following year after the foundation of Hospice, Inc.[138] Wald's panel was simply called "Hospice," and introduced the convention attendees (which mostly consisted of nurses in training and nurse educators) to the hospice as a concept, the work being done in England at St. Christopher's, and the development of Hospice, Inc., in New Haven. Her focus on historical context correlates with the lack of institutional authority hospice had at this point. The intent was to spread the word, not to educate on ideology.

From there, hospice speaking engagements exploded. In their Community Relations report from April 1974, Hospice, Inc., reported 42 talks in the period between January and April. These talks, peaking with twenty talks in the month of March, were to a variety of organizations. Fifteen were to church communities, eight to community organizations, and nineteen to healthcare professionals of various kinds, including nursing school administrators and nursing conferences. This focus on church and medical communities aligns with their goal to spread the philosophy of hospice care with those providing care to the dying, as clergymen and nurses were the frontlines of death care at this time. The outreach to community organizations was a more unique element to put their philosophy of community engagement into action. This significant informational effort – forty-two talks in four months – shows the eagerness with which Hospice began to spread their concept, and the reciprocal eagerness of the community in hearing their increasing wisdom and authority on the subject. Many of these talks were upon direct request and drew positive response.

Their prolific informational initiatives continued with a visit to Oregon in 1975 to meet with the Benedictine Sisters of Mt. Angel who founded the Providence Benedictine

[138] Anne Harvey to Florence Wald, November 13, 1973, box 20, folder 1, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Nursing Center in 1957. On a panel with the director of the Benedictine Sister's nursing home, a nurse working in the local acute care hospital, and the team leader of the local visiting nurse association, Florence and Henry Wald presented the concept of hospice and made an argument for it as the most appropriate and responsive type of care within the current available options for the terminally ill, followed by presentation from the other three panel participants. The inclusion of representatives for hospice shows a stark development in the recognition of hospice as an established care system, on par with nursing homes, acute inpatient care, and VNA care. This event is notable most prominently for the meticulous transcription and following publication, and the unique setting of direct comparison/debate between these four care delivery systems. Given the detailed account of this talk (a rare luxury), I will be analyzing this talk most closely, given the bulk of material to work with. This symposium was organized by the Gerontological Education Director of the Benedictine Sisters, emphasizing the educational focus of the event. The nurse head of the Benedictine Sisters Gerontology Team introduced the session by saying "we are all in this process together and we benefit by sharing with one another."[139] The unique makeup of the symposium speakers and the specific structure (four presentations followed by discussion and Q&A for all speakers together) allowed for a smooth exchange of ideas and put all types of care on the same level, a structure that was not used in any other symposium or conference that I encountered. This was truly, as both the director and team leader note, an opportunity for learning, reflection, and collaboration, covering a breadth of topics including the cost of these types of care, discussion of when each type would be appropriate, and intended patient demographics. Though both Walds took the opportunity to advertise their services, they had

[139] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

not yet reached a point of authority to teach others how to start their own hospice. In attendance were the sisters, all of whom were involved with caring for the elderly and sick and were very engaged with the presented material given the questions asked in the Q&A portion of the event, discussed later in more detail. The hospice portion of the talk went as follows:

The Walds start by acknowledging the context that they found themselves in, saying "as we came into Mt. Angel, we found it very different from New Haven. Its beauty; its simplicity and openness touched us. We also saw that it can take on tremendously complex problems…"[140] and went on to acknowledge the refugee and economic struggles the community had been dealing with. This attention to the context and place in which care is given was central to their philosophy from the beginning, is visible in their planning in New Haven, and is continued in their education initiatives by their discussion of the New Haven setting and the acknowledgement of the setting of the population they are speaking to. This touch of customization goes far to support the effectiveness of their helping others create similarly effective programs and facilities in their own communities.

They go on to describe the principles of hospice care, using a patient example to guide their decision-making process. To start, they display a slide image of a patient, and describe her: "This patient, as you can see from her weary eyes and strained neck muscles and taut body, is suffering from pain; pain all over. She is weak and appears to have been losing weight."[141] Possible causes of her pain include: "metastasis, pain from intercranial

[140] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[141] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

pressure, pain because the tumor itself is pressing on a sensitive organ, or it may be from infection or anxiety."[142] Thus, they introduce the concept of psycho-social assessment to treat any emotional sources of pain. This process of evaluating possible causes of her pain involves the physician, the nurses, social workers, a psychiatrist, and clergymen. This passage emphasizes three primary hospice philosophies: psychosocial integration, interdisciplinary teams, and learning from experience. By framing the example in this way, the audience are introduced to the methods of analysis, the process of treatment determination, and the common symptoms that would necessitate the treatment. In this way, their educational methodology, not only their approach to patient care, was interdisciplinary. The patient example further shows how symptoms that may be written off as normal symptoms for terminal patients are treatable within the hospice model.

Continuing to emphasize the importance of psychosocial care, they provide another patient example, this time concerned with complex interfamily dynamics instead of pain. They highlight the home care nurse's skill in noticing an unhelpful dynamic between a terminal patient and his sons. The nurse's ability to notice and articulate the problem is testament enough to the efficacy of their internal training program. The man's sons felt uninformed about his health and care plan, and the patient felt unable to talk about his declining health with his children for fear of burdening them. With the help of a social worker, the family could navigate the difficulties of having terminal illness in the home and, "relate to one another as they had in the past and bring warmth and dignity into this last

[142] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

phase of life."[143] In this case, the resolution of anxiety for both the patient and his family is the marker of successful care. This choice of example is notable for the depiction of conflict and conflict resolution, and for the equal importance placed on the patient and the patient's family. Also, the choice to include another patient narrative example is a careful and effective method of teaching, showing their skill in effectively communicating information. Inevitably, conflict arises in this difficult stage of life and with interpersonal conflict comes anxiety, often contributing to a patient's pain. With this example, the Walds were able to communicate the importance of including the family in care planning, and the benefit of having an interdisciplinary team on hand, especially to handle these difficult interpersonal conflicts.

The Walds then turn their focus to the emotional toll of this work on the staff. Reflecting the ethos shown in their internal training curriculum, the Walds emphasized the importance of staff support systems by acknowledging the impact of death on the staff, not just the patients and families. To combat this, they encouraged part time employment and hiring individuals with personal experience with death as they have, "a competence that transcends any kind of professional expertise."[144] Aligning with their original planning documents, hiring process, and employment standards, they recommend these practices to acknowledge the difficulty of this profession and maintain the emotional health of their employees.

They conclude their portion of the symposium by discussing the considerations of location and of their future inpatient building: "such criteria as density of population,

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[143] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[144] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

incidence of irreversible degenerative disease in that population, the series of transportation arteries in a network which enables patients and families to be in close."[145] The depth to which they describe the planning process shows their dedication to supporting other communities in founding their own hospice facilities that would fit the needs and use the resources available in their specific locales. In addition to these infrastructural considerations, Henry Wald describes the integration of natural light and green space into the design of the building, and the structure of the patient rooms: "we concluded that better interpersonal relationships and reassurance were best satisfied by a 4-bed ward as a module" as opposed to the single and two-bed standard in most hospitals and nursing homes.[146] Though they did not go into specifics on having arrived at the ideal 4-bed ward in this talk, the initial inspiration came from St. Christopher's hospice in London and was confirmed by the Planning Group's early study of the needs of the dying.[147] The architect of the in-patient facility went into detail in one of his articles, describing,

> A second key to community is the four-bed room. This represents a major departure from a current trend to private and semiprivate rooms… Each patient has a roommate on one side and free space on the other for families to gather. All four roommates are

[145] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[146] Manuscript "Death, Dying and Decisions III: Alternatives in Care," April 29, 1975, Box 11, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.; Lo-Yi Chan, *"A Better Place for the Dying," American Institute of Architects,* (1976): 42-45, MS 1659, box 11, folder 12, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.
[147] "Annual Report to the National Institute of Health" 1975 Hospice, Inc. box 7, folder 84; Manuscript "Progress Report - First Observations and Impressions of St. Christopher's Hospice – September 15, 1970" Edward Dobihal, box 17, folder 89, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

close enough together so that contact is easily made. The families are a little farther away and can keep the distance or close it as they choose."[148] [149]

The detail with which they spoke on these planning and design issues conveyed the importance of these spaces and considerations to their audience and gave the listeners the tools to apply the principles to their own existing or future facility. The detail further goes on to emphasize the seriousness of their endeavor and the authority they have earned via their studies of the dying, personal experience, and subsequent analysis. Though, the remaining gaps in their first-hand knowledge and continued referral to St. Christopher's revealed their newness.

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[148] Lo-Yi Chan, *"A Better Place for the Dying," American Institute of Architects,* (1976): 42-45, MS 1659, box 11, folder 12, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

[149] A Timeline of discussions of four-bed wards:

- "They need to see and hear the people. St. Christopher's Hospice in London provides this by having four people in a bay." "Hospice, Inc. – From Dream to Reality - Foundation of Thanatology and Union Theological Seminary" Florence Wald, November 4, 1972, box 20 folder 16.
- "…patient rooms will have four beds to foster communication and exchange of experiences." Hospice, Inc., Newsletter, December 1974, box 7 folder 86.
- "… terminal patients have a characteristic fear of isolation and abandonment. For this reason, most patient rooms will have four beds." Manuscript "Annual Report to the National Institute of Health," 1975, Hospice, Inc., box 7, folder 84.
- "'Twos are a disaster. When a patient has made a friend and the friend dies, the loss is traumatic enough to convince the survivor not to risk making further relationships.' Threes and fives are better but they have the geometric problem of an odd man out." Lo-Yi Chan in conversation with Sylvia Lack in "Hospice: A New Building Type to Comfort the Dying," Lo-Yi Chan, *AIA Journal*, December 1976, box 3, folder 25.
- "Also need more research on what <u>really</u> is optimum number of beds. A. how to go about studying this. B. what are we trying to measure? – patient/family, happiness, satisfaction, self-esteem, isolation as privacy…" Manuscript, "Report of Research Section of International Workgroup on Death, Dying and Bereavement," June 1978, box 15, folder 62.

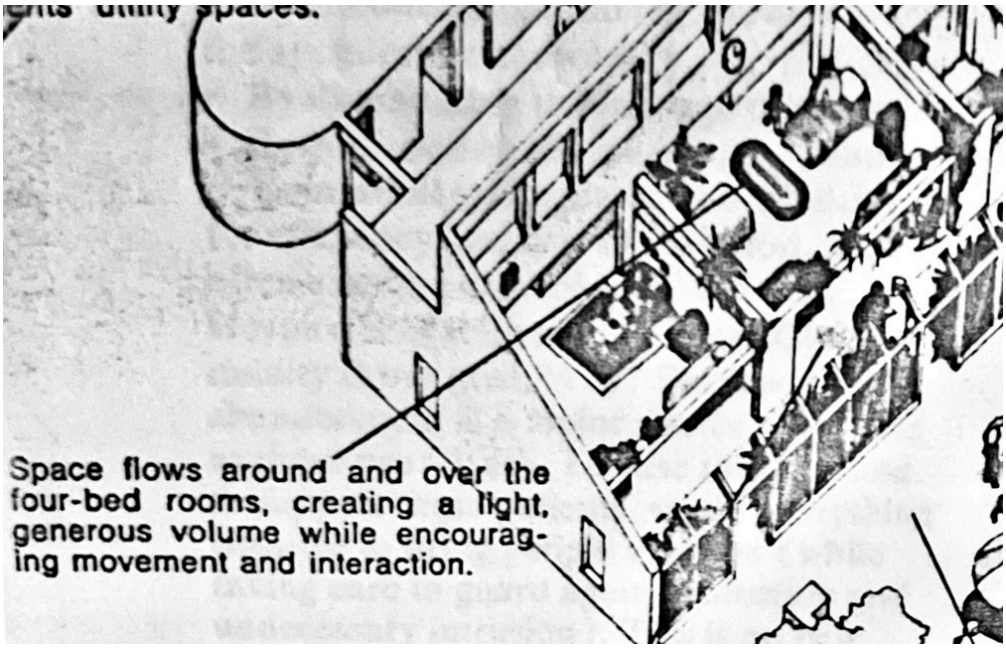

Figure 2: Partial three-dimensional rendering of CT Hospice inpatient facility

From Lo-Yi Chan, "A Better Place for the Dying," *American Institute of Architects*, (1976): 42-45, MS 1659, box 11, folder 12, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut. The four-bed room is emphasized with a leading line and the caption: "Space flows around and over the four-bed rooms, creating a light, generous volume while encouraging movement and interaction." This image is a part of a larger rendering of the inpatient facility designed by Architect Lo-Yi Chan.

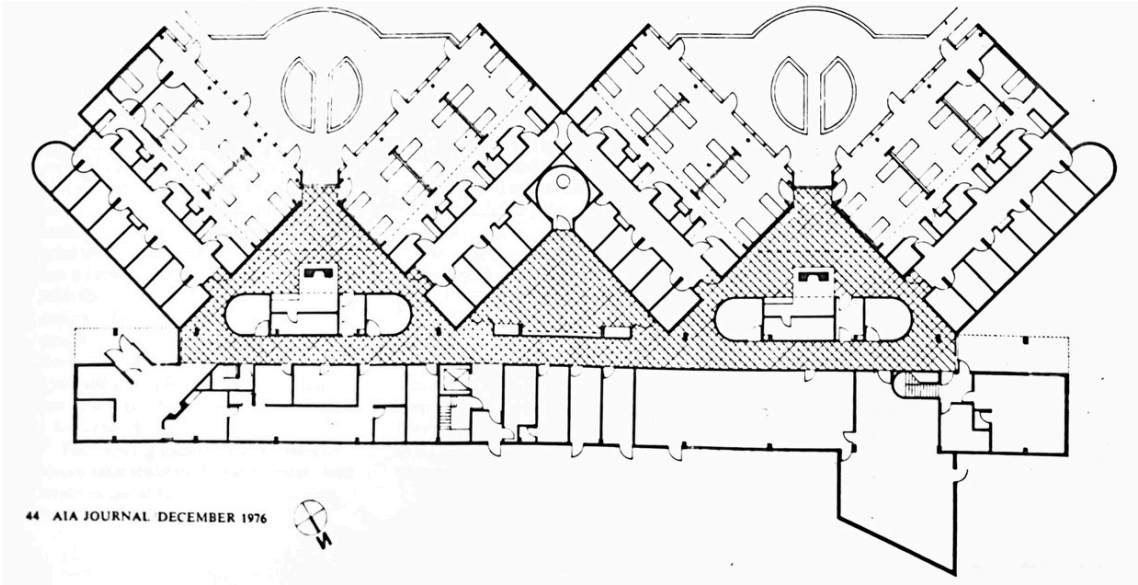

Figure 3: Partial two-dimensional drawing of CT Hospice inpatient facility

From Lo-Yi Chan, "A Better Place for the Dying," 44. The beds are shown from an aerial view as rectangles in each of the 14 rooms in the facility.

In the subsequent Q&A session, the specificity of the questions posed by the audience are a testament to their engagement in the presentation, and the way in which they will bring these principles into their own work. The panel fielded questions about the efficacy of a four person room (inconclusive in personal experience, as the inpatient facility had yet to be constructed, but successful in London), the use of pain medication (weighing the benefits of addictive drugs, taking inspiration from Saunders' 'total pain' approach), the potential benefits of meditation on outlook (unfamiliar to the Walds, but finished with a request to speak after the panel showing the collaborative nature of the event, and the incomplete nature of the Walds' knowledge), and questions about payment and funding. This last question would be one of the more prominent issues in comparison of these various care systems. At this stage of development, Hospice was still relying on NCI grants and philanthropic gifts and had not yet moved on to the exploration of third-party payers. Though this talk had the tone of advice-giving, in the specifics of the thought process of developing the in-patient, infrastructure considerations, and example patient narratives, the consistent citations of St. Christopher's in London maintained Hospice's identity as a institution still in early development. This in combination with the (friendly) debate-style panel structure, painted the picture of a growing institution with experimental knowledge to share. Hospice did not yet have the experience under their belt to recommend their approach with total authority.

The speaking engagements that Hospice participated in were a common element throughout their development, enabling them to share central pieces of their developing philosophy including: the importance of context and community, the psycho-social assessment via interdisciplinary team, family involvement, supports for the staff, and intentional physical spaces. These were communicated through a teaching ethos of learning

from experience and teaching by example. The frequency of invitations to speak underlines the efficacy and popularity of this early-emerging and constant aspect of their education initiative.

Building New Organizations to Advance Awareness

To continue Hospice's journey towards authority and establishment, Hospice foundational members contributed to the founding of the International Working Group on Death, Dying, and Bereavement in 1974. This group mainly consisted of thanatologists (professionals in the study of understanding and coping with death and dying), but also included Hospice members, Cicely Saunders and her colleagues at St. Christopher's, Balfour Mount (a Canadian hospice and palliative care pioneer), Bill Worden (a prominent psychologist in grief studies), Mary Vachon (Canadian palliative care nurse), and Sylvia Lack (future primary physician of Hospice, Inc.). This team worked tirelessly over the following four years to create a standards-of-care document to, "delineate the standards of care essential for dying patients, the role of their families and the support system for the care givers." This document, published in 1979 in the *American Journal of Nursing*, *Journal of the American Medical Association*, and the *Lancet*, outline principles including: respecting the patient's values, providing constant and easily accessible care, addressing "total pain", and supporting the patient's family through bereavement.[150] This document would go on to influence government processes of accreditation for hospice and palliative care facilities.[151]

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[150] "ASSUMPTIONS AND PRINCIPLES UNDERLYING STANDARDS FOR CARE OF THE TERMINALLY ILL," American Journal of Nursing, February 1979, IWG Publications, International Work Group on Death, Dying, and Bereavement, https://www.iwgddb.com/wp-content/uploads/2021/12/1979-Original-1979-Assumption-Principles.pdf.

[151] Legislation, "SUMMARY OF H.R.12358 Hospice Care Standards Introduced April 25, 1978," April 1978, box 17, folder 101, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

The formation of this organization only furthered Hospice, Inc.'s establishment in being a primary source of knowledge about hospice care, and expanded their ability to work interdisciplinarity with experts in parallel fields.

However, the fight between regulation and innovation would continue to be negotiated within the hospice/terminal illness care community. Although standards and regulations were necessary for the implementation of these care practices throughout the country, there was a balance to be struck. In an address to the National Hospice Organization meeting in 1978, Cicely Saunders declared that, "there must be a balance between standards and flexibility." She went on to say that, "the only accreditation that really matters is the respect from, understanding of, and integration with the whole medical scene."[152] This was a perspective that would continue to influence Hospice education efforts going forward.

Hospice Education in Academic Curricula

Hospice, Inc., was dedicated to education from the beginning, as can be seen in their internal training plans and speaking engagements. Going beyond their first-hand ability to communicate knowledge, Hospice, Inc., also created curriculum for external use, notably for the Yale School of Nursing. This marks an important milestone in their institutional authority, as it gave them the power to spread their philosophy and practices beyond their limited manpower. Though Hospice, Inc., would not be the organization to ultimately institute hospice and palliative care curriculum in professional schools, they did succeed in introducing core principles of hospice care into professional curriculums by way of

[152] Manuscript, "ALTERNATIVES IN CARE FOR THE TERMINALLY ILL THE IMPLICATIONS FOR NURSING EDUCATION," Florence Wald, 1978, box 20, folder 2, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

cancer/oncology courses. This marks another example of the entanglement between oncology and hospice care, and the way that this entanglement with a respected field assisted in the hospice model gaining widespread authority.

Curriculum work for external institutions started earlier in the development of Hospice than one might expect. In 1974, Hospice, Inc., signed a contract with the Yale School of Nursing in 1974 decreeing that Hospice, Inc., would provide courses on oncological nursing for a group of trainees chosen by the Yale School of Nursing. These courses would include "core" classes, as well as several specialized courses and one course for faculty. It is important to note the content of these courses as oncological in focus, not palliative or hospice. Seeing that Hospice, Inc., had barely started their Home Care program running at this time, their own institutional knowledge was not established enough to begin spreading, especially in an institution as prestigious as the Yale School of Nursing. However, this contract marks the beginning of Hospice, Inc.'s identity as a capable teaching entity and source of knowledge for future healthcare workers. Given their grant from the National Cancer Institute around the same time, it further establishes hospice identity as a cancer care service.

Even without explicit hospice content in this initial curriculum, the tenants of hospice philosophy are evident in the course titles. One titled, "Care of the Dying Cancer Patient and His Family," clearly emphasizing the importance of the family in making care decisions, aligns with the central hospice idea of treating the family as a unit being enacted at that time in the emerging Home Care program. In her scribbled notes for this class, Wald outlines the basic principles she wishes to cover, including: "patient/family is like a mobile; therefore

patient/family is in need of care."[153] This delicate metaphor communicates the careful balance that is the family dynamic in these times of illness. This aspect of patient care is the most central principle that her notes bring repeatedly, revealing the significance for her. The repeated emphasis points to an aspect of nurse training that is lacking. Another course included in this curriculum: "Physical Care of the Patient with Advanced Cancer," subtly recognizes the shift in priorities for those in advanced stages of disease.[154] The careful word choice of "physical" creates the idea that there are other important aspects of care. Though not explicitly stated, the emotional and spiritual aspects of care emphasized in hospice practices are implicit. Even without the explicit permission to spread the hospice knowledge, Wald and her colleagues found a way to include them in the ethos of the core cancer courses.

The question remains as to why Hospice, Inc., was employed to create a series of courses about cancer patients, especially lacking the 'terminal' determination. In addition to the history between hospice and cancer diagnoses being the primary terminal illness, there was an emerging movement towards understanding the psychodynamic conflicts patients experienced with disease, cancer specifically. Susan Sontag's focus on the emotional aspects of cancer in, *Illness as Metaphor,* published in 1978, highlights the broad pattern of societal associations between disease and morality. Her personal journey with breast cancer and feelings of shame and guilt surrounding her diagnosis highlight the profound impact of this moral rhetoric.[155] The curriculum focus on cancer as the primary terminal illness is supported

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[153] Document, "Notes for Yale School of Nursing class," Florence Wald, March 17, 1975, box 19, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[154] Contractual agreement, "Contractual Agreement - Yale School of Nursing and Hospice, Inc. - ONCOLOGY NURSING PROGRAM," Yale University School of Nursing, 1974, box 19, folder 4, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.; Eleanor Taggart to Florence Wald, March 14, 1975, box 19, folder 5, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.
[155] Susan Sontag, *Illness as Metaphor* (Farrar, Straus, Giroux, 1988).

by the growing contemporary conversation surrounding cancer as an emotional disease and the past entwinement with hospice.

Before hospice principles could be integrated into professional education, support, funds and staff needed to be acquired. Hospice advocated for these provisions in a document called, "Alternatives in Care for the Terminally Ill: The Implications for Nursing Education," addressed to the Department of Health, Education, and Wellness in 1978. In this document, Wald asserts that, "decisions will have to be reached about what is now included in a nurse's professional education, what will be and should be included, and, finally, what hospices--and other services for the terminally ill--will themselves, in all probability, have to contribute to staff training."[156] She goes so far as to put a time limit on her recommendation, saying: "Within the next five years, there needs to be a mechanism for all involved professionals to have options in learning and in work experience relating to the care of those facing terminal illness."[157] This demand for education reform also came with a slightly contradictory maintenance of freedom: "each hospice unit needs the freedom to develop its own service -- as it relates to the needs of its own community – and an environment that fosters interdisciplinary sharing of work and decision-making." Once again, she recommends, this time explicitly, the need for community customization and interdisciplinary teams. However, this balance between not regulating the facilities but bringing the principles of these facilities into education programs was a fine one. Many might argue that although they may have been able to maintain this balance initially, it has now become distinctly unbalanced.

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[156] Manuscript, "ALTERNATIVES IN CARE FOR THE TERMINALLY ILL THE IMPLICATIONS FOR NURSING EDUCATION," Florence Wald, 1978, box 20, folder 2, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

[157] Manuscript, "ALTERNATIVES IN CARE FOR THE TERMINALLY ILL THE IMPLICATIONS FOR NURSING EDUCATION," Florence Wald, 1978, box 20, folder 2, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

After years of development and advocacy, Hospice successfully spread their philosophy throughout the country, with over 200 fully functioning programs by 1978.[158] Though it would take some more time for hospice and palliative care programs to be instituted in professional training, eventually they would become commonplace in both medical and nursing school.

Conclusion

In addition to establishing the Home Care program and opening the inpatient facility, Hospice, Inc., was on a journey of intellectual development. Their education initiative, present in every step of their institutional development, shows the ideological inspirations, lessons learned, and developing authority. Their intentions and educational methodology are visible in the details of their talk notes, curriculum drafts, and correspondence. The steadily diminishing emphasis on historical context and St. Christopher's practices traces this intellectual development, and shows Hospice, Inc., establishing themselves as not only a functioning facility providing direct patient care, but also a model for other future American Hospices. Their educational initiatives grew and ultimately established their authority in the creation of hospice facilities and ongoing practice in the United States.

[158] Hospice, Inc., Newsletter, April 1978, box 7 folder 86, MS 1659, Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

Conclusion

The Hospice Medicare benefit went live in November 1983, covering hospice care for people enrolled in Medicare. This piece of legislation also created patient criteria to qualify for hospice care and a system of certification and reimbursement for hospice facilities. After proving to be cost effective and widely supported, Congress made the benefit permanent in 1986. With this development, hospice care became a fixture of the American medical system rather than an experiment within it. In 1999, Connecticut Hospice transitioned away from the facility designed and built for them, and into a larger water-front property where they remain today. The larger building allowed them to expand their inpatient capacity from 44 to 52 beds, and to support a larger care and administrative staff. Today, Connecticut Hospice remains a staple of the New Haven community. On any given day, Connecticut Hospice will treat an average of 134 patients, according to recent Medicare census.[159] Of these patients, 56% reside at home, and 37% are in nursing homes, assisted living facilities, or skilled nursing facilities.[160] Their inpatient facility is rarely at capacity, reflecting the continued importance of the Home Care program in their model of care, even today.

Connecticut Hospice remains an important source of hospice knowledge by teaching workshops on hospice care, pain management, and the psychosocial aspects of end-of-life care; welcoming medical students to shadow or complete clinical hours in the facility; supporting advanced degree candidates in nursing, social work, pharmacy, music therapy, and art therapy with mentorship and experience with interdisciplinary care teams; and by

[159] "The Connecticut Hospice, Inc - 2022-2024," Medicare.Gov, Center for Medicaid and Medicare Services, 2024, https://www.medicare.gov/care-compare/details/hospice/071500/view-all?state=CT.
[160] Center for Medicaid and Medicare Services, "The Connecticut Hospice, Inc - 2022-2024."

supporting palliative care research with the John D. Thompson Institute for Education, Training, and Research.

However, the hospice industry has departed from the ideals of Connecticut Hospice, especially regarding finances. Connecticut Hospice is a non-profit hospice, representing a small sect in the, majority for-profit, hospices in the country. This is a recent trend: as of 2022, three-quarters of Medicare-certified hospice programs in the country are for-profit, compared with only 5% in 1990.[161] The for-profit sector of hospice care is a sharp deviation from the original model pioneered by Hospice, Inc. This is a part of the mounting critiques of the American hospice system which overall has the reputation of a money-grabbing industry "rife with exploitation," according to Ava Kofman, author of "The Hospice Hustle."[162] This article, published in 2022, investigated the staggering Medicare fraud plaguing the hospice industry and the country. Following the outrage, the Center for Medicaid and Medicare Services (CMS) overhauled hospice inspection rules. However, financial exploitation of the Medicare benefit has remained widespread.

The consequences of hospice fraud extend far beyond financial losses. When enrollment in hospice becomes a revenue strategy rather than a medical decision, the patient pays the price. Patients go undertreated, under-visited, and too often abandoned in their final days, ironically recreating the very conditions that prompted the modern hospice movement in the first place. The dominance of the for-profit model has invited abuse and restructured the incentives of end-of-life care. The vulnerable patient population makes the industry

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[161] Center for Medicaid and Medicare Services, "The Connecticut Hospice, Inc - 2022-2024."; Melissa D. Aldridge et al., "National Hospice Survey Results," *JAMA Internal Medicine* 174, no. 4 (2014): 500–506, https://doi.org/10.1001/jamainternmed.2014.3.
[162] Ava Kofman, "The Hospice Hustle," *ProPublica*, November 28, 2022, https://www.propublica.org/article/hospice-healthcare-aseracare-medicare.

particularly at risk of exploitation. This is not simply a regulation issue; this is a crisis of care. Until the financial structure of end-of-life care is reckoned with, the movement that set out to care for the dying risks becoming a cautionary tale about what happens when compassion becomes an industry. American hospice was built to center the dying patient. It's time to return to that.

Annotated Bibliography

Primary Sources

Archival Materials

Florence and Henry Wald Papers (MS 1659). Manuscripts and Archives, Yale University Library.

The Florence and Henry Wald Papers collection include eighteen linear feet of meticulously organized papers detailing the professional careers of both Florence and Henry Wald. Included are unpublished internal documents from the planning and founding of Hospice, Inc., research notes and draft materials of the "Interdisciplinary Study of the Dying," newspaper clippings from both local and national papers, promotional materials from other hospice groups, considerable correspondence, and materials from Florence Wald's time at the Yale School of Nursing. This archive gave me huge insight into the inner workings of Hospice, Inc., and the development of their model of care. I use materials from this archive as the foundation for much of my argument in this thesis, both using the materials at face value for understanding the detailed timeline of events and understanding institutional relationships, but also using the documents as images in themselves to tell the story of the minute thought processes behind the foundation of Hospice, Inc.

Books and Manuscripts

Kübler-Ross, Elisabeth. *On Death and Dying*. Routledge, 1973. https://doi.org/10.4324/9780203010495.

Upon its publication, this book by Elisabeth Kübler-Ross gained rapid fame for her articulation of the five stages of grief. However, this book contains much more insight into the culture of death and dying of this time beyond the overused and oversimplified stages of grief theory. This book served as my introduction to studies of the dying and provided important basis for comparison in my analysis of Wald's study of the needs of the dying. Kübler-Ross's questions and points of interest in her investigation revealed the prominent issues of death and dying during the 1950s and 1960s.

Stoddard, Sandol. *The Hospice Movement: A Better Way of Caring for the Dying*. With Internet Archive. New York : Vintage Books, 1978. http://archive.org/details/hospicemovement00stod.

This book rides the line between primary and secondary source, as it is the first analytical account of the modern hospice movement, but it was published in 1978, before the CT Hospice inpatient unit was even built. This dual function proved useful in that it gave me insight into the perception of the modern hospice movement at the time when it was fresh, but from a more objective and analytical perspective than the media coverage, for instance. It

also provided me with further detail on the personal experiences that called people to work in hospice, as Stoddard herself had a somewhat traumatic experience with a terminal patient that cemented her interest and faith in the hospice model of care.

Journal Articles

Cassell, Eric J. "Dying in a Technological Society." *The Hastings Center Studies* 2, no. 2 (1974): 31–36. https://doi.org/10.2307/3527480.

This article gave me a bioethicists perspective on the increasing medicalization of terminal illness and death, as well as providing insight into the other aspects of American society that contributed to the changes. These included changing family structures, the impact of the larger statistical concept of life expectancy on the individual, and the increasing importance in distinguishing death due to old age versus death due to illness. This article was fundamental in my understanding of the cultural influences impacting the context within which hospice emerged.

O'gorman, Stella Mary. "Death and Dying in Contemporary Society: An Evaluation of Current Attitudes and the Rituals Associated with Death and Dying and Their Relevance to Recent Understandings of Health and Healing." *Journal of Advanced Nursing* 27, no. 6 (1998): 1127–35. https://doi.org/10.1046/j.1365-2648.1998.00659.x.

While not central to my argument, this article provided me with insight into the ongoing debate happening within the field of nursing surrounding the balance of emotional and physical treatment of symptoms in end-of-life and death. The author argues for the necessity of a multicultural understanding of death rituals and coping mechanisms before treatment or advice can be given. This acknowledgement of ritual and the diversity of cultural approaches to death was not something I saw emphasized in many earlier documents, making this an interesting outlier.

Newspaper and Magazine articles

Auclair, Paula. "Director at Hospice To Address Women: East Haddam." *Hartford Courant*, February 19, 1978. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/545205760/55DF22C632CD45D8PQ/11?accountid=11311&sourcetype=Newspapers.

Day, Mary-June. "Morbid But Necessary." *Hartford Courant*, February 4, 1973. ProQuest Historical Newspapers: Hartford Courant.

https://www.proquest.com/hnphartfordcourant/docview/551413949/fulltextPDF/F4780B80542A4EECPQ/77?accountid=11311&sourcetype=Newspapers.

Eastland, Dorothy. "State Now Center For 'Death' Advice." *Hartford Courant*, November 16, 1974. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/552384362/fulltextPDF/F4780B80542A4EECPQ/40?accountid=11311&sourcetype=Newspapers.

Einhorn, Linda. "Hospital Group Gets $200,000 in Grants." *Hartford Courant*, March 25, 1973. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/551446547/fulltextPDF/F4780B80542A4EECPQ/37?accountid=11311&sourcetype=Newspapers.

Einhorn, Linda. "When She Died, Paula Was Ready." *Hartford Courant*, March 18, 1973. ProQuest Historical Newspapers: Hartford Courant.

Finhorn, Linda. "New Hospital Concept Seeks To Attach Dignity to Death." *Hartford Courant*, March 18, 1973. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/551468675/F4780B80542A4EECPQ/42?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Cancer Unit Seeks Funds." May 1, 1975. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/543931531/fulltextPDF/F4780B80542A4EECPQ/52?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Discussion Will Focus On Hospice Services." May 1, 1977. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/544913113/55DF22C632CD45D8PQ/9?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Doomed Cancer Patients Given Heroin, LSD for Pain, Anxiety." November 13, 1971. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/550993484/fulltextPDF/F4780B80542A4EECPQ/65?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Facilities To Aid Dying Planned." October 28, 1975. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/544133465/fulltextPDF/F4780B80542A4EECPQ/39?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Garden Club Sponsors Hospice Program Talk: Old Lyme." May 14, 1979. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/545720379/55DF22C632CD45D8PQ/16?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Home Care Offered for Seriously Ill." March 22, 1974. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/552105168/fulltextPDF/F4780B80542A4EECPQ/31?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Agent To Address Cancer Society." January 9, 1975. https://www.proquest.com/hnphartfordcourant/pagepdf/542765042/Record/F4780B80542A4EECPQ/29?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Agent Will Speak About Cancer: Thomaston." February 16, 1977. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/544860849/fulltextPDF/49DF4B3E98754FE7PQ/4?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Facility Given $20,000." January 13, 1976. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/544399083/F4780B80542A4EECPQ/25?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Is Called No Nursing Home." April 7, 1976. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/544523153/F4780B80542A4EECPQ/21?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice May Open By 1976." April 24, 1976. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/552228836/F4780B80542A4EECPQ/24?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Opens Fund Drive for 44-Bed Facility." December 17, 1974. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/pagepdf/552436085/Record/F4780B80542A4EECPQ/30?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Raises Half of Goal." December 9, 1975. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/544225600/F4780B80542A4EECPQ/23?accountid=11311&sourcetype=Newspapers.

Hartford Courant. "Hospice Receives $125,000 Grant From Foundation." June 11, 1974. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/552228411/F4780B80542A4EECPQ/26?accountid=11311&sourcetype=Newspapers.

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Hartford Courant. "Terminal Cancer Home Gains $50,000 Grant." July 19, 1973. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/551694572/fulltextPDF/F4780B80542A4EECPQ/53?accountid=11311&sourcetype=Newspapers.

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Rhinelander, David. "Facility for Dying Stalled by Review." *Hartford Courant*, March 14, 1974. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/docview/552155727/fulltextPDF/F4780B80542A4EECPQ/35?accountid=11311&sourcetype=Newspapers.

Rhinelander, David. "State Panel Said To Back Center for Terminally Ill." *Hartford Courant*, June 6, 1974. ProQuest Historical Newspapers: Hartford Courant. https://www.proquest.com/hnphartfordcourant/docview/552246592/fulltextPDF/F4780B80542A4EECPQ/33?accountid=11311&sourcetype=Newspapers.

Rogoway, Rick. "Hospice Aide Confident as Talks End." *Hartford Courant*, May 21, 1974. ProQuest Historical Newspapers. https://www.proquest.com/hnphartfordcourant/pagepdf/552214559/Record/F4780B80542A4EECPQ/27?accountid=11311&sourcetype=Newspapers.

The digital archive of the Hartford Courant was an excellent source of local media coverage for the entire breadth of the development of Hospice, Inc. The Hartford Courant is a very old newspaper with a solid presence in Connecticut, but also a broader recognition across the country with several

breakout articles that made national news. This local focus was extremely helpful in my understanding of the logistical media coverage of CT Hospice development. Florence Wald did not keep clippings about funding updates or zoning updates, and this digital archive filled that gap in the media coverage for me.

Altman, Lawrence . "Doctors' Training Subject of Study." *The New York Times*, November 23 1975. MS 1659, box 11, folder 10, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

Boodman, Sandra. "The Explosive Growth of Hospices." July 1978. MS 1659, box 11, folder 13, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

Colen, B. D. "Death With Dignity – At Home." *The Washington Post,* November 1975. MS 1659, box 11, folder 10, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

Schmeck Jr., Harold. "Addictive Drugs as a Way of Easing Death." November 1977. MS 1659, box 11, folder 10, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

The Washington Post. "The Hospice Movement." February 1978. MS 1659, box 11, folder 12, Florence and Henry Wald Papers, Manuscripts and Archives, Yale University, New Haven, Connecticut.

Though I found these five articles in the clipping folders of the Florence and Henry Wald Papers, I include them in this section because of their importance and accessibility to those without access to the Wald archive. These articles, mainly feature articles, added significant depth to my understanding of the perception of death and hospice care in this critical period for CT Hospice. Colen's article "Death With Dignity at Home" was especially useful due to the focus on CT Hospice and the included patient narratives.

Secondary Sources

Connecticut Hospice

Abel, Emily K. *Prelude to Hospice: Florence Wald, Dying People and Their Families*. Critical Issues in Health and Medicine. Rutgers University Press, 2018.

A historical analysis of the study that Florence Wald and her colleagues conducted on the needs of dying patients and their families, focusing on the major characters and their personal motivations. This is the prominent

historiography of CT Hospice, and I engage with Abel's work and extend the analysis to the features of the hospice that these characters created, pulling from the same major archive.

Cost of Caregiving

Abel, Emily K. "A 'Terrible and Exhausting' Struggle: Family Caregiving During the Transformation of Medicine." *Journal of the History of Medicine and Allied Sciences* 50, no. 4 (1995): 478–506. https://doi.org/10.1093/jhmas/50.4.478.

Covers the experiences of family caregivers in the 19th and 20th centuries with a focus on the shift from home-based to hospital-centric care that happened during this time. This text gave me a deep understanding of the physical, emotional and financial burden of caregiving, and the relationship between that burden and the larger medical landscape.

Harold Braswell, PhD. *The Crisis of US Hospice Care*. Johns Hopkins University Press, 2019. https://doi.org/10.1353/book.67474.

A current-day exploration of the burden of care that still rests on families' shoulders in the current hospice model of care. Braswell takes a historical perspective to examine the evolution of hospice's reliance on the familial caregiver, and the resulting stresses on family and familial relations. Additionally, he covers the options for dying patients without familial or hired caregivers, and the neglect that follows. Braswell finished by proposing a new model of care to address these unsustainable practices.

English Hospice and Cicely Saunders

Clark, David. *A Bit of Heaven for the Few? : An Oral History of the Modern Hospice Movement in the United Kingdom - Harvard University*. Lancaster: Observatory, 2005. https://hollis.harvard.edu/primo-explore/fulldisplay?docid=01HVD_ALMA212099534120003941&context=L&vid=HVD2&lang=en_US&search_scope=everything&tab=everything.

Prominent hospice historian, David Clark, offers a first-hand account of the modern hospice movement in the UK in the form of an oral history. This account gives incredible detail into the specifics of daily practices, experimentation with pain management techniques, and the culture of St. Christopher's Hospice.

Clark, David. "An Annotated Bibliography of the Publications of Cicely Saunders–1: 1958–67." *Palliative Medicine* 12, no. 3 (1998): 181–93. https://doi.org/10.1191/026921698678023443.

Cicely Saunders was a prolific author throughout her life. David Clark's annotated bibliography guided me in approaching her vast body of work, and appreciating the most relevant of her works to my study of American Hospice.

Clark, David. *Cicely Saunders – Founder of the Hospice Movement: Selected Letters 1959–1999*. Oxford University Press, 2005. https://doi.org/10.1093/acprof:oso/9780198569695.001.0001.

David Clark presents an accessible volume of Cicely Saunders' personal correspondence, complimented with brief summaries of her achievements, personal life, and professional work for each period of her life. These letters give invaluable perspective into Saunders' personality and her approach to her life's work. The accompanying exposition provided critical details of her education, varied professional careers, and development of St. Christopher's Hospice.

Francis, Ara. "Death Positivity, White Respectability: A Critical History of Cicely Saunders and the Hospice Model of Care." *Critical Sociology*, June 26, 2025, 08969205251350396. https://doi.org/10.1177/08969205251350396.

This new article provided me with valuable insight into the ongoing discussion of the modern hospice movement, and the growing body of work criticizing the beatification of Saunders and Wald and recognizing the gaps in their perspective of cultural diversity and socioeconomic impact on health outcomes.

Nursing Professionalization

Reverby, Susan M. *Ordered to Care: The Dilemma of American Nursing, 1850–1945*. Cambridge University Press, 1987.\

This text traces the long tension between nursing as a duty and a calling, and a paid profession. Starting with pre-World War I nursing, Reverby comments on the issues of modern nursing stemming from the contradictions in the value of care at the origins of nursing as a professional field.

Rosenberg, Charles E. *The Care of Strangers: The Rise of America's Hospital System*. Johns Hopkins University Press, 1995.

This book describes the shift from the hospital as a welfare institution for the poor, to a technologically advanced center of professional and technical progress, with nursing being the constant. This understanding was critical for

my understanding of the foundational role of nurses in American medicine, and the increasing medicalization of death.

Medicalization of Death

Bell, Susan E., and Anne E. Figert. "Medicalization and Pharmaceuticalization at the Intersections: Looking Backward, Sideways and Forward." *Social Science & Medicine* 75, no. 5 (2012): 775–83. https://doi.org/10.1016/j.socscimed.2012.04.002.

This article defines medicalization and the newer concept of pharmaceuticalization and provided me with an understanding of the framework behind the concept of medicalization. This supported my understanding of the arguments surrounding the medicalization of death in the 19th and 20th centuries.

Tatum, Paul E., Kevin W. Craig, Karla T. Washington, and Debra Parker Oliver. "Getting Comfortable With Death: Evolution of the Care of the Dying Patient." *Missouri Medicine* 111, no. 4 (2014): 298–303.

This article covers the changing treatment of dying patients from the 1970s to the time of publication, and the corresponding medical care structures that supported and or responded to this rapidly changing care. Though not strictly historiography, this text also gave me insight into the local development of hospice in Missouri in the 1980s.

Death in America

Abel, Emily K. *The Inevitable Hour*. Johns Hopkins University Press, 2017.

This text was hugely foundational in my understanding of medical understanding of death from the late 19th century to 1965, right before the modern hospice movement. Tracing the shift in care being rendered in the home to institutional care, this book both served as my core understanding of death in America, and Abel's foundation for her later study of Florence Wald and subsequent book, *Prelude to Hospice*.

Clark, David, and Gara LaMarche. *Transforming the Culture of Dying: The Work of the Project on Death in America*. Oxford University Press, 2013.

The Project On Death in America (PDIA) emerged in 1994 as a parallel movement to the modern hospice movement. David Clark analyzes the influences on this movement, including the impact of funding and policy on emerging healthcare reform. This book gave me perspective on the other

efforts to address end-of-life care in America in the second half of the 20th century and gave me tools to explore the influence of funding and policy on the development of CT Hospice.

Pain management

Bourke, Joanna. *The Story of Pain: From Prayer to Painkillers*. First edition. Oxford University Press, 2014.

Knight, Helen P., and Richard E. Leiter. "Hospice at Home: Historical Perspectives and Their Implications for End-of-Life Care Today." *Journal of Pain and Symptom Management* 67, no. 5 (2024): e625. https://doi.org/10.1016/j.jpainsymman.2024.02.056.

Scarry, Elaine. *The Body in Pain: The Making and Unmaking of the World*. Oxford University Press, 1985.

The history of pain and pain management was my introduction to the concept of hospice, so I would be remiss to exclude these formative texts from my sources. The Scarry and Bourke texts were my introduction to the study of pain as a historical concept, forever shaping my interest in the history of medicine. The Knight article provides a perspective of hospice focused on the development of pain management techniques and the concept of 'total pain'.

Structures of Care

Clark, David. *To Comfort Always: A History of Palliative Care*. Oxford University Press, 2016.

This book gives a comprehensive history of end-of-life care, starting in the 19th century with the idea of a good death and tracing it through to the contemporary development of the specialized palliative medicine field in the 1970s.

Starr, Paul. *The Social Transformation of American Medicine*. Basic Books, 1982.

This book covers the rise of professional sovereignty in American medicine. This included boundaries between physicians and other healthcare providers (i.e. nurses, etc.) in the early 20th century. This guided my understanding of the establishment of institution and specialization in hospice and end-of-life care.

For-profit Hospice

Kofman, Ava. "The Hospice Hustle." *ProPublica*, November 28, 2022. https://www.propublica.org/article/hospice-healthcare-aseracare-medicare.

This provocative article covers the journalistic investigation and semi-oral history of the prolific hospice Medicare fraud problem in America. Covering a variety of issues from neglect, false diagnoses, overprescription of opioids, exploitation of rural populations, and fraudulent business practices, this article revealed the dark world of for-profit hospice to the public.